

# Parenting and family interventions in lower and middle-income countries for child and adolescent mental health: A systematic review

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## ABSTRACT

**Background:** Given the protective effect of nurturing caregivers and families for child and adolescent mental health, there is a need to review and synthesize research evidence regarding the effectiveness of parenting and family interventions in low and middle-income countries, including humanitarian settings. To advance practice, further understanding of the active ingredients of such interventions and implementation factors that lead to effectiveness are essential.

**Method:** This systematic review, an update from a previous review, included studies on any parenting or family intervention for children and adolescents aged 0–24, living in a low- or middle-income country, that quantitatively measured child or adolescent mental health outcomes. We searched Global Health, PubMed, PsychINFO, PILOTS and the Cochrane Library databases on the 9th July 2020, and updated on the 12th August 2022. Risk of bias was assessed using an adapted version of the NIH Quality Assessment Tool. We extracted data on: effectiveness outcomes, practice elements included in effective interventions, and implementation challenges and successes.

**Main findings:** We found a total of 80 studies ( $n = 18,193$  participants) representing 64 different family or parenting interventions, 43 of which had evidence of effect for a child or adolescent mental health outcome. Only 3 studies found no effect on child, adolescent or caregiver outcomes. The most common practice elements delivered in effective interventions included caregiver psychoeducation, communication skills, and differential reinforcement. Key implementation strategies and lessons learned included non-specialist delivery, the engagement of fathers, and integrated or multi-sector care to holistically address family needs.

**Preliminary conclusions:** Despite a high level of heterogeneity, preliminary findings from the review are promising and support the use of parenting and family interventions to address the wider social ecology of children in low resource and humanitarian contexts. There are remaining gaps in understanding mechanisms of change and the empirical testing of different implementation models. Our findings have implications for better informing task sharing from specialist to non-specialist delivery, and from individual-focused to wider systemic interventions.

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## 1. Introduction

There is a growing evidence base for mental health and psychosocial support (MHPSS) interventions for children and adolescents in low resource and humanitarian settings [1], however there has been limited research evaluating family interventions in these contexts despite strong evidence for the role critical role that families play in safeguarding child and adolescent mental health [2], and for the effectiveness of parent and family interventions for a wide range of mental health and child protection outcomes in high-income countries and non-humanitarian contexts [3].

Children and adolescents affected by humanitarian crises or other adversities have been consistently found to have increased risks of mental disorders, internalizing and externalizing symptoms and psychological distress [4]. This distress has been linked to experiences of extreme poverty, inequality, war, violence, displacement and loss, but also to daily stressors associated with poor living conditions and pervasive uncertainty and insecurity [5]. In the context of major adversities, children's mental health and resilience is associated with family functioning, parental mental health, supportive interpersonal relationships, and community-level factors, which interact in complex, context-specific ways [6,7]. Whilst mental health services have historically tended to focus on individual children and their internal mechanisms of coping, recent focus has accordingly shifted towards their broader social ecology including families, peers, teachers and communities [8]. Models of psychological distress have begun to incorporate social and interpersonal processes into the theoretical understanding of how human beings respond to major stress [9], recognizing the role of these factors in promoting mental health, and preventing or treating mental disorders.

The effectiveness of psychosocial interventions in low-resource and humanitarian settings is mixed. An individual patient meta-analysis of studies of psychological support for children indicated some evidence of efficacy for post-traumatic stress disorder (PTSD) but limited evidence for depression and anxiety [1]. An umbrella review of meta-analyses of psychosocial interventions in LMICs found weak evidence for effectiveness of approaches with children and adolescents [10]. Individual reviews have highlighted a lack of studies for family and community-level interventions, as well as a bias towards specialized cognitive behavioral interventions [11–14]. A recent meta-analysis of parenting interventions in conflict-affected settings found very few studies, and mixed effects, calling for greater investment in this area [18,15]. Further, international guidance across LMIC has focused primarily on parenting intervention with little synthesis available on family interventions that address the broader family system [16]. Service providers and humanitarian organizations have made calls to action, highlighting the need for community-based parenting and family interventions (e.g. the WHO Commission on Social Determinants of Health [17,18]), and establishing the effectiveness of these interventions has been identified as a key research priority for the MHPSS in humanitarian settings) [19].

An earlier systematic review of evaluations of parenting and family MHPSS interventions in low- and middle-income countries [20] found preliminary evidence for children and adolescent outcomes, particularly for mental health and wellbeing, as well as parenting and family functioning. An analysis of practice elements using Practice Wise [21] identified that common intervention components of effective interventions included caregiver components (such as praise, coping skills, modeling behaviors) and family components with children and caregivers together (such as communication, support networking, and insight building), as well as session structure (such as assigning homework, facilitator relationship building, and promoting access). The review highlighted the need for more rigorous trials, and greater attention to implementation factors and mechanisms of change.

With attention recently shifted towards parenting and family approaches, several new interventions have been developed that target

parenting (e.g. the Caregiver Support Intervention in Lebanon [22]) and family processes (e.g. Tuko Pamoja in Kenya [23]), and this evidence has not yet been captured in existing reviews. At the same time, a need to understand the *implementation* process, including the mechanisms of change, feasibility, and acceptability, not just the effectiveness of interventions, has also been increasingly highlighted as a priority in the field [19,24,25]. This understanding can help to inform the adaptation, delivery, scaling and sustainment of effective interventions in different contexts and populations.

To gain an up-to-date understanding of the state of the evidence regarding these priorities (i. parent and family interventions, ii. implementation factors), we updated the systematic review conducted by Pedersen et al. [20] on the effectiveness of parent *and* family MHPSS interventions in LMIC, including analysis of practice elements, and with the addition of a synthesis of implementation factors and tested mechanisms of change of included interventions. The review aimed to provide a comprehensive synthesis of evidence for the effects and implementation components of parent and family interventions. As available evidence is scarce, we included studies with a range of methodologies including pre-post, quasi-experimental, and randomized control trial (RCT) designs. Given the known challenges with conducting research in humanitarian contexts, we included all studies conducted in low- and middle- income countries, where the vast majority of conflict and forced displacement occurs [26], and where similar resource constraints and social determinants of poor mental health are prevalent. The research questions were 1) what is the current state of the evidence for family or parenting interventions leading to child or adolescent mental health or wellbeing outcomes in low- and middle-income countries? 2) what are the common practice elements in interventions that have an effect?, 3) what are the reported theoretical underpinnings and mechanisms of change in these interventions, and 4) what do we know about implementation factors important for achieving desired impact, scale and sustainment.

## 2. Method

A systematic review was conducted drawing on the Cochrane Handbook for Systematic Reviews (Version 5.1.0), and is reported here according to the Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA) criteria [27] (see checklist in supplementary materials). The protocol for the review was published in the International Prospective Register of Systematic Reviews (PROSPERO) (reg: CRD4202018820). One deviation from the protocol was made, to exclude early childhood development interventions from the practice and implementation component analyses due to major differences in intervention aims and structure (i.e. not focused only on mental health outcomes). The review used multiple sources of data to identify academic papers, manuals and intervention materials of any family systemic or parenting intervention, that has been delivered with children, adolescents, and/or their parents in low- or middle-income countries. As this review was an update of a previous review [20], the same databases, search terms, and inclusion and exclusion criteria were used covering the years since the last review. However, data on mechanisms of change and implementation were also extracted drawing on the implementation checklist by Tierney et al. [28]. Authors were contacted to gain additional information, including to obtain intervention manuals and training curriculum, as well as to obtain authors' views on challenges and lessons learned. When authors did not respond we drew on the available published data.

### 2.1. Search strategy

The following databases were searched: Global Health (Ovid), PubMed (Ovid), PsychINFO (Ovid), PILOTS (ProQuest), and Cochrane Library (Wiley Online Library). Search terms were identical to the previous review, with the first and senior author of the previous review

(GAP and FB) consulted on the search process to ensure each step was identical. The search terms in full are listed in Supplementary Table 1. Search terms included synonyms for the population, the intervention and the outcome, such as “low income country” AND “family intervention\*” AND “mental health.” Search terms were entered as keywords for all text, with the exception of PubMed which was limited to a title/abstract search. All databases were searched on 9th July 2020, and updated on the 12th of August 2022. Searches were restricted to 2017-present, as the previous review by Pedersen et al. [11] covered 1967–2017. Searches were not restricted by language.

A grey literature search was also conducted. Eight international mapping exercises of family or parenting interventions known to the research team were screened for eligible interventions and evaluations (see Supplementary Table 2). Authors of all included published studies were also contacted to enquire about any additional publications or unpublished reports. We included published and unpublished reports of challenges and lessons learned for included evaluated interventions.

## 2.2. Eligibility criteria

Inclusion and exclusion criteria were:

- 1) Population: children and adolescents, defined as any person younger than age 24 in order to include children, adolescents and youth as per the World Health Organization's definition; and living in a low- or middle-income country, which were defined as all countries on the World Bank list [17]) of low-and middle-income countries for the 5 years leading up to the included studies' data collection.
- 2) Intervention: any family, systemic, or parenting intervention for the target population and setting, in any format, and at every level of the IASC (Inter-Agency Standing Committee) four-tiered pyramid model for mental health and psychosocial support in emergencies [29]. Interventions were identified by the authors/service provider's description of the intervention or theoretical underpinnings, or by dual examination of the intervention description and content. Interventions that were child-focused, or included families or parents purely for logistical reasons or just to share child-focused content, were excluded. Parent-focused interventions were defined as any intervention involving a parent/caregiver, which aimed to improve outcomes for the children and parent/caregiver. For example, it may have addressed parenting skills, relationship building, or managing one's own emotions in the context of parenting. Family-focused interventions were defined as any intervention involving parents or caregivers and/or the family unit which aims to improve outcomes for members of the family. For example, by improving family engagement, relationships, and effectiveness in handling adversity (poverty, disorder or illness, daily stressors, disaster, conflict, etc.). Family was broadly defined, and not limited to biological or immediate family members, in line with the previous review by Pedersen et al. [20].
- 3) Comparison: we included quantitative studies with assessments at pre- and post-intervention, with and without a comparison group.
- 4) Outcome: quantitative assessment of child or adolescent mental health or wellbeing.

## 2.3. Study selection

Records obtained from searches were exported to EndNote 20, and each was independently screened for inclusion by two members of the research team (AM and SF), using title, then abstracts and then full text. Percentage interrater agreement was 83.4% (Cohen's  $k = 0.66$  indicating substantial agreement), with all disagreements resolved through discussion and consensus (AM, SF, TB, FB).

## 2.4. Data extraction

Data were extracted independently in full by two members of the research team (AM and SF) using a preset matrix for the 1) population, 2) study including design, sample size, outcome measures and findings, 3) setting, including identifying humanitarian settings using the United Nations definition [30], 4) intervention including number of sessions, parenting or family sessions, format, location, and facilitators, 5) underlying theory, 6) mechanisms of change or mediational analyses, 7) core elements and activities, and 8) implementation components (a) acceptability, b) attendance, c) adoption, d) appropriateness, e) fidelity, d) implementation costs, e) intervention complexity, f) penetration, g) reach, h) sustainability, and i) challenges and lessons learned. Duplicate extractions were merged, and discrepancies resolved through a third research team member (TB). For implementation data, contact details for the authors/service providers of included studies were used to contact authors by email in order to request access to the manuals or other intervention materials, and lessons learned in the development or delivery of the intervention.

## 2.5. Quality assessment

AM conducted a quality assessment of the evidence supporting each included intervention, checked by a second rater (SF), with discrepancies resolved through discussion with a third rater (TB) and agreed by consensus. The same procedure was used as the recent review by Pedersen et al. [20], using NIH Quality Assessment Tool for Before-After (Pre-Post) Studies With No Control Group [31], adapted to include additional questions for RCTs from the Jadad Scale for Reporting Randomized Controlled Trials [32].

## 2.6. Data synthesis

The synthesis of data revolved around: 1) intervention effectiveness, 2) practice components, 3) theoretical underpinnings and proposed mechanisms of change, and 4) implementation factors. We organized our narrative synthesis of included studies and findings according to study design (with controlled trials and pre-post non-control evaluations summarized separately), intervention (family, parenting), and age group (child- or adolescent-focused). We were not able to conduct meta-analyses due to high heterogeneity in intervention type and outcomes measured.

### 2.6.1. Practice element coding

We used PracticeWise [21] to code intervention content descriptions, as well as manuals when they were available. In line with the review by Pedersen et al. [20], and due to the systemic nature of included interventions, we added sub-coding for whether the practice element was directed to children, their caregivers or both. Only interventions with evidence of effect for at least one child or adolescent mental health outcome (assessed either pre-post or compared with a control group) were coded. Due to major differences in the aims and structure of early interventions compared to child or adolescent interventions, we decided to focus the practice element coding analysis on childhood and adolescence, and excluded early childhood development and infant-focused interventions. Practice components were coded by two independent raters (split between AM, FB and TB). Components were then ordered by the frequency of interventions that use them (highest to lowest).

### 2.6.2. Implementation component analysis

Implementation data of the same child and adolescent interventions with evidence of an effect were analyzed using Implementation Component Analysis [19,33] by one research team member (TB) and a proportion were duplicate checked by a second team member (SF), with discrepancies resolved through discussion and consensus. Implementation descriptions for each of the ten implementation components (e.g.

acceptability, feasibility, sustainability) were searched and extracted from each study. Codes were then reviewed for themes, and themes then organized across studies. The most commonly described themes (3 studies or more) were presented.

### 3. Results

#### 3.1. Study selection

A total of 2351 studies were retrieved from the five academic databases searched and 240 reports were retrieved from the grey literature mapping exercise screening (See Fig. 1 for the PRISMA diagram). Based on titles, 333 were included for abstract review, and 58 of these were included for full text review. Out of these, 34 studies were identified for inclusion, 8 additional studies were added after screening the reference lists of included studies, and 2 eligible studies were sent to us by the authors of included papers. Reasons for exclusion included that there was no evaluation, no child mental health outcome or the study location was in a high-income country. Forty-five authors from the previous and updated review responded to our request for additional information, providing additional studies for review, manuals or materials, or summaries of challenges and lessons learned. Two manuals were in Spanish, and three were in Arabic. All other manuals were available in English. For the grey literature search, 250 reports were screened but no additional evaluated interventions were identified. In addition to the 36 studies covering 28 interventions identified by the original review by Pedersen et al. [20], a total of 80 studies with a total of 18,193 participants, covering 64 different family or parenting interventions, were included in this review.

[Fig. 1 flowchart insert here].

#### 3.2. Study characteristics

##### 3.2.1. Study locations and settings

Included studies were conducted in Sub-Saharan Africa ( $n = 23$ ), East Asia and Pacific ( $n = 14$ ), the Middle East and North Africa ( $n = 12$ ), Latin American and Caribbean ( $n = 11$ ), Europe and Central Asia ( $n = 10$ ) and South Asia ( $n = 10$ ). Twelve studies were conducted in humanitarian settings at the time of data collection. Forty studies delivered the intervention in a community setting, followed by a hospital or primary health care facility ( $n = 12$ ), homes ( $n = 7$ ), home and school ( $n = 5$ ) school only ( $n = 4$ ), a mental health service ( $n = 4$ ), or a private setting ( $n = 1$ ), and the remainder did not report the setting.

##### 3.2.2. Study design

As shown in Table 1, a total of 42 studies included parent-focused interventions, which were parent or caregiver sessions without children present, and delivered to groups of caregivers or individual parents. Parent-focused intervention studies used RCT ( $n = 27$ ), pre-post ( $n = 11$ ), and non-randomized control trial ( $n = 4$ ) methodologies. The other 38 studies included family-focused interventions, which consisted of whole family sessions including both caregivers and children or adolescents, and were delivered to groups of families (multi-family sessions) or individual families. Family-focused intervention studies also used RCT ( $n = 20$ ), pre-post ( $n = 16$ ), and non-randomized control trial ( $n = 2$ ) methodologies.

[Table 1 insert here].

##### 3.2.3. Study quality

Risk of bias was rated as high (score of 10–12), medium (score of 9), or low (score of under 9). Out of the 80 studies included, 25 were rated as high, 17 as medium and 38 as low. The majority of studies had clear

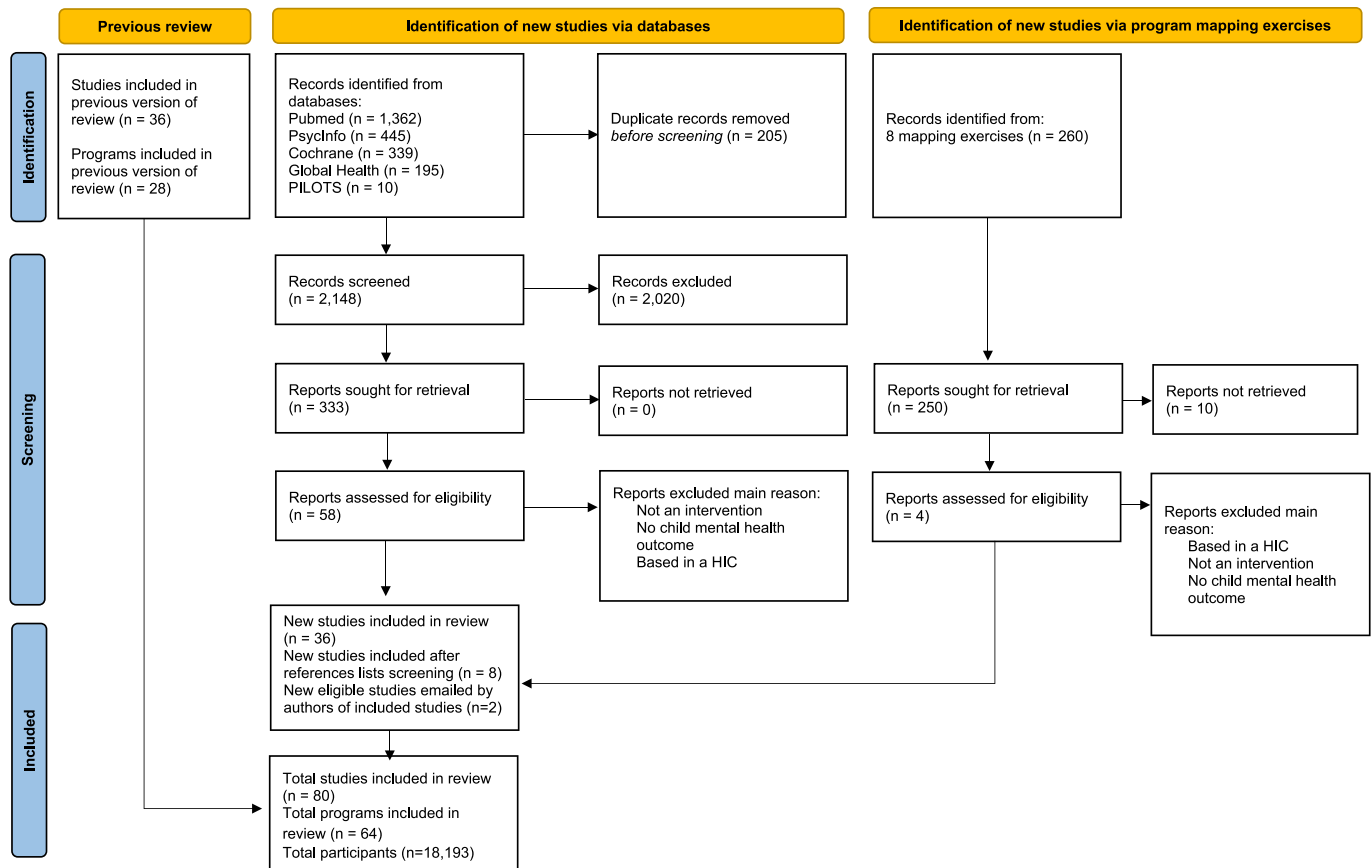


Fig. 1. PRISMA 2020 flow diagram for updated systematic reviews which included searches of databases, registers and other sources.

**Table 1**Description of included studies ( $N = 80$ ) of parent and family interventions for child mental health in LMICs.

| Study; location                                       | Sample size; age group            | Intervention type; setting; therapist type                                                     | Number of sessions; session format                                                                                          | Comparison group                                        | Parent/caregiver/ family outcomes                                                                                                                                                                                                                                                  | Child outcomes                                                                                                                                                                                                                                                                                                    |
|-------------------------------------------------------|-----------------------------------|------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Parent-focused interventions: RCTs                    |                                   |                                                                                                |                                                                                                                             |                                                         |                                                                                                                                                                                                                                                                                    |                                                                                                                                                                                                                                                                                                                   |
| Abedi & Vostanis (2010)<br>Esfahan, Iran              | $N = 49$ ; children & adolescents | Quality of Life Therapy; mental health care setting; psychologist                              | 8; groups of caregivers                                                                                                     | Waitlist                                                | Quality of Life <sup>a</sup> (QoLI)                                                                                                                                                                                                                                                | Obsession and compulsion severity <sup>a</sup> (CY-BOCS); life satisfaction - global <sup>a</sup> , family <sup>a</sup> , school <sup>a</sup> , friends, self and living environment <sup>a</sup> (BMSLSS); anxiety <sup>a</sup> (RCMAS)                                                                          |
| Aghebati et al. (2020)<br>Tehran, Iran                | $N = 300$ mothers; children       | Group “Triple P” Level 4; not reported; trained psychologist                                   | 8; 5 group sessions and 3 personal telephone contacts                                                                       | Waitlist                                                | Dysfunctional discipline <sup>a</sup> , depression anxiety & stress <sup>a</sup> (DASS-42), care & overprotection/control <sup>a</sup> (PDI)                                                                                                                                       | Child behavioral problems <sup>a</sup> (CBCL)                                                                                                                                                                                                                                                                     |
| Aghebati, Gharraee, Shoshtari & Gohari (2013)<br>Iran | $N = 30$ ; children               | Group “Triple P” Level 4; medical setting; not reported                                        | 8; 5 group sessions, 3 individual telephone sessions                                                                        | Waitlist                                                | Parenting <sup>a</sup> (PS), depression and anxiety <sup>a</sup> (DASS), parental bonding <sup>a</sup> (PBI)                                                                                                                                                                       | Child behavior and emotional problems <sup>a</sup> (CBCL)                                                                                                                                                                                                                                                         |
| Bangirana et al. (2021)<br>Kampala, Uganda            | $N = 120$ ; caregiver-child dyad  | “Creating opportunities for parent empowerment (COPE)” <sup>a</sup> ; hospital; psychologist   | 3 phases; parents of children in intensive care                                                                             | Psychoeducation only                                    | Maternal depression (HSCL)                                                                                                                                                                                                                                                         | Child report - behavioral problems (SDQ), Parent report - behavioral problems (CBCL)                                                                                                                                                                                                                              |
| Cooper et al. (2009)<br>South Africa                  | $N = 342$ ; infants               | Home-visiting; home; lay facilitators                                                          | 16; individual                                                                                                              | Routine care                                            | Maternal sensitivity and intrusiveness <sup>a</sup> , depression <sup>a</sup> (clinical interview + EPDS)                                                                                                                                                                          | Infant attachment <sup>a</sup> (strange situation)                                                                                                                                                                                                                                                                |
| Dehkordian et al. (2017)<br>Iran                      | $N = 60$ ; children & adolescents | Mindful parenting; not reported                                                                | 8; individual                                                                                                               | Social thinking intervention, exercise, no intervention | None                                                                                                                                                                                                                                                                               | Quality of life questionnaire <sup>a</sup> (PedsQL), ADHD (CPRS)                                                                                                                                                                                                                                                  |
| Diken et al. (2017)<br>Turkey                         | $N = 24$ ; children               | “First Steps to Success”; school & home; counselor, teacher & parent                           | 20–30 days teacher with student (first 5 days modeled by the counselor), 6 weeks parents with child (at least 15 min a day) | No intervention                                         | Qualitative only                                                                                                                                                                                                                                                                   | Problem behaviors, social skills, and academic competence <sup>a</sup> (Turkish SSRS)                                                                                                                                                                                                                             |
| Dybdahl & Pasagic (2000)<br>Tuzla Region, Bosnia      | $N = 87$ ; children               | Psychosocial intervention program plus medical treatment; Community setting; Preschool Teacher | 20 (weekly for 5 months); combination of groups and individual caregiver group                                              | Medical care only                                       | Mother’s well-being: today, usually <sup>a</sup> , maternal PTSD (IES) <sup>a</sup> ; weight/height <sup>a</sup> , weight/age <sup>a</sup> , hemoglobin                                                                                                                            | Cognitive performance (RCPM), depression (BDI), child problems: anxiety and sadness, withdrawal, psychosomatic complaints subscales, concentration, social support: trust, advice <sup>a</sup> , help subscales, child’s well-being: today, usually, interviewer observation of problems <sup>a</sup> , resources |
| Kousha & Abbasi Kakrodi (2019)<br>Rasht, Iran         | $N = 60$ ; children & adolescents | “Triple P”; not reported; child and adolescent psychiatrist & psychiatry co-resident           | 8; groups of mothers                                                                                                        | Routine care                                            |                                                                                                                                                                                                                                                                                    | Paediatric quality of life <sup>a</sup> (QoLI)                                                                                                                                                                                                                                                                    |
| Hamadani et al. (2006)<br>Bangladesh                  | $N = 206$ ; infants               | “Psychosocial Stimulation”; community setting; trained lay counselor                           | 28 and 39 ( $M = 33.6$ $SD = 3.6$ ) group meetings with mothers and 43 to 83 ( $M = 68.3$ $SD = 8.7$ ) home visits          | Nutritional support only, better-nourished children     | Mother’s knowledge of nutrition, health and hygiene, child development and child-rearing practices <sup>a</sup>                                                                                                                                                                    | The mental and psychomotor development (BSID-II), behavior rating <sup>a</sup>                                                                                                                                                                                                                                    |
| Husain et al. (2021)<br>Kallar Syedan, Pakistan       | $N = 774$ ; infants               | “Learning through Play Plus (LTP+)” <sup>a</sup> ; home; community health workers              | 10; group sessions                                                                                                          | Treatment as usual                                      | Maternal depression <sup>a</sup> (PHQ-9), anxiety <sup>a</sup> (GAD-7), quality of life <sup>a</sup> (EuroQoL-5), maternal social support <sup>a</sup> (MSPSS), caring environment <sup>a</sup> (HOME), parenting <sup>a</sup> (KAP), parenting competence <sup>a</sup> (Parenting | Child development <sup>a</sup> (ASQ)                                                                                                                                                                                                                                                                              |

(continued on next page)



Table 1 (continued)

| Study; location                                         | Sample size; age group                      | Intervention type; setting; therapist type                                                                                                                | Number of sessions; session format                    | Comparison group                        | Parent/caregiver/ family outcomes                                                                                                                                                                                                                                                                                | Child outcomes                                                                                                                                                                              |
|---------------------------------------------------------|---------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------|-----------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Gardener et al. (2005)<br>Jamaica                       | N = 114; infants                            | “Psychosocial Stimulation”; home; trained community health worker                                                                                         | 6–24 per month; home visits                           | Zinc supplementation only, routine care | Sense of Competence Scale)<br>None                                                                                                                                                                                                                                                                               | Mental development* (GMDS)                                                                                                                                                                  |
| Guo et al. (2016)<br>Shanghai, China                    | N = 81; children                            | Triple P group parenting programme; community setting; accredited triple P facilitator                                                                    | 8; combination of groups and individual caregiver     | Waitlist                                | Parenting practices*, parent adjustment*, family relationships, parental teamwork (PAFAS), parenting self-efficacy* (CAPES), parenting practices in academic context* (PCACQ), satisfaction with academic achievement*, perception of child's academic learning (PPCAA), positive parenting- child report (APQ)* | Emotional & behavioral problems* (CAPES), problem behaviors in academic context* (APBQ), learning stress (LSQ), academic self-regulation, child report <sup>b</sup> & parent report (SRQ-A) |
| Lachman et al. (2021)<br>Taguig, Philippines            | N = 120; early childhood                    | “Masayang Pamilya Para Sa Batang Pilipino” Parenting Programme (“Happy Family for Filipino Children” MaPa); community centers; non-specialist facilitator | 12; group sessions                                    | Treatment as usual                      | Maltreatment* (ISPCAN), positive parenting (PARYC), parental attitude* (PS), parenting efficacy (PSOC-ES), parenting stress (PSI), mental health (DASS), wellbeing (WHO-5), parenting dependency on alcohol, intimate partner violence (CTS2S), marital satisfaction (KMSS)                                      | Child behavior problems* (ECBI), socio-emotional development (ASQ)                                                                                                                          |
| Maselko et al. (2022)<br>Pakistan                       | N = 889; early childhood                    | Thinking Healthy Program, Peer-delivered Plus (THPP+); community; non-specialists                                                                         | 18; group and individual sessions                     | Enhanced usual care                     | Depression (PHQ-9), disability (DAS)                                                                                                                                                                                                                                                                             | Socioemotional development (SDQ), developmental milestones (BSITD III)                                                                                                                      |
| Mehrin et al. (2022)<br>Kishorganj district, Bangladesh | N = 785; infants                            | “Group Reach-Up and Learn” parenting sessions; community clinic; community health care workers                                                            | 52; group sessions                                    | Clinic as usual                         | Parenting knowledge*, stimulation at home* (FCI)                                                                                                                                                                                                                                                                 | Child development* (BSITD), child behavior* (Wolke's Behavior Rating Scale)                                                                                                                 |
| Mejia et al. (2015)<br>Panama City, Panama              | N = 108; children                           | Triple P parent discussion group; community setting; accredited triple                                                                                    | 1; groups of caregivers                               | No intervention                         | Depression*, anxiety*, and stress* (DASS-21)*; parenting style - laxness, hostility*, over-reactivity (PS)                                                                                                                                                                                                       | Behavioral difficulties parent report-problems*, intensity* (ECBI)                                                                                                                          |
| Miller et al. (2022)<br>Tripoli, Lebanon                | N = 240; children                           | “Caregiver Support Intervention (CSI)”;<br>community; non-specialist provider                                                                             | 9; group sessions (mothers and fathers separately)    | Waitlist                                | Parenting (DPS), parental warmth, responsiveness and harsh parenting* (DPS), parenting knowledge* (DPS), psychological distress (K10), wellbeing (WEMWBS), stress, stress management                                                                                                                             | Child psychosocial wellbeing* (Kid-KINDL parent report)                                                                                                                                     |
| Miller et al. (2020)<br>Tripoli, Lebanon                | N = 78 families (151 individuals); children | “Caregiver Support Intervention (CSI)”;<br>community; non-specialist provider                                                                             | 9; group sessions (mothers and fathers separately)    | Waitlist                                | Caregiver stress*, stress management*, parenting*, caregiver psychosocial wellbeing* (WEMWBS), caregiver psychological distress* (K10)                                                                                                                                                                           | Child psychosocial wellbeing* (Kid-KINDL parent report)                                                                                                                                     |
| Ozyurt, Gencer, Ozturk & Ozbek (2016)<br>Turkey         | N = 50; children                            | Group “Triple P”; not reported; trained psychologist                                                                                                      | 8; 5 group sessions & 3 individual telephone sessions | Waitlist                                | Anxiety* (STAI), general wellbeing* (GHQ)                                                                                                                                                                                                                                                                        | Anxiety related emotional disorders* (SCARED child and parent report), functioning and severity* (CGAS), severity* (CGI—S), internalizing and                                               |

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Table 1 (continued)

| Study; location                                                                          | Sample size; age group          | Intervention type; setting; therapist type                                                       | Number of sessions; session format                    | Comparison group                                           | Parent/caregiver/ family outcomes                                                                                                                                                             | Child outcomes                                                                                                                         |
|------------------------------------------------------------------------------------------|---------------------------------|--------------------------------------------------------------------------------------------------|-------------------------------------------------------|------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------|
| Puffer et al. (2015)<br>Lofa Country, Liberia                                            | N = 270; children               | Parents Make the Difference—parent skills building; community setting; lay health worker         | 10; combination of groups and individual caregivers   | Waitlist                                                   | Harsh discipline* (MICS), positive behavior management* (own scale), caregiver-child interactions caregiver report*, child report* (own scale), caregiver-child communication (DPICS-III)     | externalizing problems* (SDQ)<br>Cognition (cognitive battery); emotional symptoms, conduct problems, hyperactivity/inattention (SDQ)  |
| Sangawi, Adams & Reissland (2018)<br>Iraqi Kurdistan                                     | N = 16; early adolescents       | “Systematic Training for Effective Parenting (STEP)”; school-setting; mental health professional | 7; groups of mothers                                  | No intervention                                            | Parenting styles*, parental stress* (PSS), participants knowledge about parenting*                                                                                                            | Academic self-concept (Myself-as-a-Learner Scale), internalizing and externalizing behavior problems (SDQ)<br>Child intensity* (CAPES) |
| Shaw et al. (2021)<br>Kuala Lumpur, Malaysia                                             | N = 137; early childhood        | Parenting intervention; community; paraprofessional                                              | 8; group sessions                                     | Waitlist control                                           | Positive parenting, inconsistent discipline, poor supervision (APQ), family functioning* (FFS), emotional distress* (RHS)                                                                     | Child development* (ASQ—C)                                                                                                             |
| Shi et al. (2020)<br>China                                                               | N = 168; infants                | Parenting intervention integrated in primary healthcare; medical setting; specialist             | 2; group sessions                                     | Routine care                                               | None                                                                                                                                                                                          | Child development* (ASQ—C)                                                                                                             |
| Sumargi et al. (2015)<br>Surabaya, Indonesia                                             | N = 143; children               | Triple P parenting seminar series; community setting; accredited triple P facilitator            | 3; groups of caregivers                               | Waitlist                                                   | Parenting practices* parent adjustment*, family relationships, parental teamwork (PAFAS); parenting style-laxness*, over-reactivity* (PS), parenting self-efficacy* (CAPES)                   | Emotional problems, behavioral problems* (CAPES)                                                                                       |
| Valades et al. (2021)<br>El Salvador                                                     | N = 64; infants                 | “Crianza Temprana”; home; community health workers                                               | 16; individual                                        | Basic obstetric and paediatric care                        | Maternal sensitivity*                                                                                                                                                                         | Attachment (Still Face)*                                                                                                               |
| Yousafzai et al. (2014); Yousafzai (2015)<br>Naushero Feroze district of Sindh, Pakistan | N = 1489; mother-infant dyads   | Responsive stimulation programme*; community setting; lay health worker                          | 24; combination of group & individual family          | Treatment as usual (routine health and nutrition services) | Mother-child interaction* (OMCI); quality of caregiving environment* (HOME-I); maternal caregiving knowledge & practices* (UNICEF-MCIS); maternal depressive symptoms <sup>d,e</sup> (SRQ-20) | Cognitive*, language*, & social-emotional* skills (BSITD)                                                                              |
| Yusuf, Gonka & Aynur (2019)<br>Turkey                                                    | N = 48; children                | Group “Triple P” Level 4; medical setting; not reported                                          | 8; 5 group sessions & 3 individual telephone sessions | Waitlist                                                   | Family functioning* (FAD), parental attitude* (PARI)                                                                                                                                          | Internalizing and externalizing behavior* (SDQ), global functioning and severity* (CGAS), severity* (CGI—S), ADHD* (ADHD-RS-IV)        |
| Parent-focused interventions: Non-randomized controlled trial                            |                                 |                                                                                                  |                                                       |                                                            |                                                                                                                                                                                               |                                                                                                                                        |
| Davis et al. (2005)<br>Serbia and Montenegro <sup>c</sup>                                | N = 141; parent-infant pairs    | European Early Promotion Project parenting program; primary healthcare setting; community nurses | 3; individual parent-infant                           | Routine care                                               | Mother mental health problems; father mental health problems; parent relationship; mother enjoyment of the child*; self-esteem, family relationships (Family Grid FG); parenting stress (PSI) | Temperament (ICQ); behavioral problems (BSQ); mental development (BSIMD)                                                               |
| Jordans et al. (2013)<br>Republic of Burundi                                             | N = 120; children & adolescents | Psychoeducation; community setting; lay health worker                                            | 2; groups of caregivers                               | Waitlist                                                   |                                                                                                                                                                                               | Aggressive behavior* (AQ); depressive symptoms (DSRS); perceived family support (A-SCAT adapted)                                       |
| Othman et al. (2010)<br>Northeast coast, Malaysia                                        | N = 79; children                | Psychoeducation program; hospital setting;                                                       | 4; groups of caregivers                               | Waitlist                                                   | Knowledge on childhood cancer (KA); perceived acquired knowledge*                                                                                                                             | Behavioral problems (SDQ)                                                                                                              |

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Table 1 (continued)

| Study; location                                                       | Sample size; age group                                 | Intervention type; setting; therapist type                                                                    | Number of sessions; session format                                         | Comparison group | Parent/caregiver/ family outcomes                                                                                                                                                                                                                      | Child outcomes                                                                                                                                                                                        |
|-----------------------------------------------------------------------|--------------------------------------------------------|---------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------|------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                                                       |                                                        | Psychologist in training & oncologist                                                                         |                                                                            |                  | (KA-p); anxiety (SA, subscale of STAD); symptoms of stress (SQ); activities with children (PA)                                                                                                                                                         |                                                                                                                                                                                                       |
| Rempel et al. (2020)<br>Vietnam                                       | N = 733;<br>infants                                    | "Parenting teamwork"; community setting; health worker                                                        | 5; 1 group prenatal session, 1 prenatal homevisit, 3 post-natal homevisits | Not reported     | Partner breastfeeding support behaviors - mother & father report*, exclusive breastfeeding*, parental relationship quality - mother & father report*                                                                                                   | Infant developmental status*                                                                                                                                                                          |
| Parent-focused interventions: Pre to post studies                     |                                                        |                                                                                                               |                                                                            |                  |                                                                                                                                                                                                                                                        |                                                                                                                                                                                                       |
| Bello-Mojeed et al. (2016)<br>Lagos, Nigeria                          | N = 20;<br>children & adolescents                      | Parent-delivered behavioral intervention; mental healthcare setting; Psychiatrist                             | 5; groups of caregivers                                                    |                  | Mother's knowledge of behavioral management of aggression* (KBMAQ)                                                                                                                                                                                     | Aggression* and self-injury* (ASIQ)                                                                                                                                                                   |
| Fayyad et al. (2010)<br>Beirut and South Lebanon, Lebanon             | N = 49;<br>children & adolescents                      | Arabic Adaptation of "Helping Challenging Children"; community/ health centers; non-specialist health workers | 8; individual or group formats with mothers                                |                  | Parenting attitudes*                                                                                                                                                                                                                                   | Externalizing and internalizing behaviors* (SDQ)                                                                                                                                                      |
| Izadi-Mazidi, Riahi & Khajeddin (2015)<br>Iran                        | N = 16;<br>children                                    | Group "Cognitive Behavioral Therapy (CBT)"; education center; clinical psychologist/ psychiatrist             | 7; groups of mothers                                                       |                  | Parenting stress* (PSI-short form)                                                                                                                                                                                                                     | Child difficulties (subscale PS)                                                                                                                                                                      |
| Lakkis et al. (2020)<br>Refugee camps in Jordan and Lebanon           | N = 124<br>parents (mother & fathers); early childhood | "SANAD (SUPPORT)" Parenting Intervention; not reported; psychologist                                          | 21; group sessions                                                         |                  | Well-being* (WHO-5), parenting stress* (PSI short form), parental discipline* (DSQ)                                                                                                                                                                    | Externalizing and internalizing behaviors* (SDQ)                                                                                                                                                      |
| Macdonald & Doostgharin (2013)<br>Tajikistan, Kyrgyzstan & Kazakhstan | N = 190<br>families; children                          | "Families and Schools Together (FAST)"; schools; professionals from health, education & social care           | 8; groups of families                                                      |                  | Family cohesion*, expressiveness, and conflict* (FES), parent-child relationship*, parents' social relationships with the community* (SRS), parental involvement with school* (PIEQ), parent's social support* (SSS), reciprocal parent support* (RPS) | Internalizing and externalizing problems* (SDQ)                                                                                                                                                       |
| Matos et al. (2006)<br>Puerto Rico                                    | N = 10;<br>children                                    | Parent-child interaction therapy; mental healthcare setting; trainee psychologist                             | 16; individual family                                                      |                  | Stressful family experiences* (FEI)*; parenting practices* (PPI)                                                                                                                                                                                       | Hyperactivity*, oppositional defiant symptoms* (DBRS); hyperactivity* & aggression* (BASC subscales); externalizing symptoms*, aggression* (CBCL); behavioral problems- number* and intensity* (ECBI) |
| Ozbek, Gencer & Mustan (2019)<br>İzmir Nüfusu, Turkey                 | N = 54;<br>children                                    | Group "Triple P" Level 4; medical setting; accredited provider                                                | 8; 5 group sessions & 3 individual telephone sessions                      |                  | Parental attitudes* (PARI)                                                                                                                                                                                                                             | Externalizing and internalizing problems* (SDQ)                                                                                                                                                       |
| Pitillas et al. (2020)<br>Freetown, Sierra Leone                      | N = 43;<br>mothers                                     | "Strong Bonds, Strong Pikin (SBSP)"; community; professional facilitators                                     | 6; group sessions                                                          |                  | Maternal sensitivity* (Ainsworth Rating Scale), parenting stress* (PSI), violent discipline* (MICS)                                                                                                                                                    | Child problems*                                                                                                                                                                                       |
| Shiri, Pouretemada, Fathabadic & Narimanid (2020)<br>Iran             | N = 17; early childhood                                | Family-based management of behavioral excesses in young children with ASD; not reported                       | 12; 10 group sessions, 1 homevisit, 1 follow-up session                    |                  | Parental self-efficacy* (PSAM), parenting stress* (PSI - short form)                                                                                                                                                                                   | Intelligence (Stanford-Binet intelligence scale), adaptive behavior* (VABS), functioning (CGI-I), repetitive behavior* (RBS-R & DORBA)                                                                |
| Solis-Camara et al. (2000)<br>Mexico                                  | N = 82 mother child pairs                              | STAR parenting psychoeducation program; community setting; lay health worker                                  | 4*; groups of caregivers                                                   |                  | Parenting behavior-nurturing*, discipline*, expectations* (PBC)                                                                                                                                                                                        | Behavioral problems* (BSQ)                                                                                                                                                                            |

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Table 1 (continued)

| Study; location                                                      | Sample size; age group                           | Intervention type; setting; therapist type                                                                         | Number of sessions; session format                                                          | Comparison group   | Parent/caregiver/ family outcomes                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | Child outcomes                                                                                                                                                                                                                                                                                              |
|----------------------------------------------------------------------|--------------------------------------------------|--------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------|--------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Yu et al. (2015)<br>Shanghai, China                                  | N = 19;<br>children                              | Behavioral family therapy;<br>community setting;<br>Psychologist                                                   | 5; individual family                                                                        |                    | Caregiver-child<br>interaction- positive*,<br>intrusive*, compliance*,<br>attention* (BCS)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   | Aggressive behaviors*<br>(CBCL)                                                                                                                                                                                                                                                                             |
| Zu et al. (2019)<br>China                                            | N = 16;<br>children &<br>adolescents             | Psychoeducational<br>intervention; medical<br>setting; psychologist                                                | 12; groups of<br>mothers                                                                    |                    | Depression* (BDI),<br>anxiety (BAI), mothers'<br>emotional word usage*                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | Autism severity* (CARS)                                                                                                                                                                                                                                                                                     |
| Family-focused interventions: RCTs                                   |                                                  |                                                                                                                    |                                                                                             |                    |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |                                                                                                                                                                                                                                                                                                             |
| Puffer et al. (2017);<br>Sim et al. (2016)<br>Tak Province, Thailand | N = 479;<br>children &<br>adolescents            | "Happy Families"<br>parenting and family skills<br>training program;<br>community setting; lay<br>health worker    | 12; groups -<br>separate parts for<br>children and parents<br>& joint parents +<br>children | Waitlist           | Child-report- parent-<br>child relationship<br>quality* (PBI); parent-<br>child warmth/affection*<br>negative relationship<br>quality* (PAR-Q);<br>discipline practices<br>(MICS); positive<br>discipline, negative<br>discipline (DI); family<br>cohesion*, family<br>communication*,<br>negative family<br>interactions* (BFFS).<br>Caregiver-report -<br>parent-child relationship<br>quality* (PBI), parent-<br>child warmth/affection*<br>negative relationship<br>quality* (PAR-Q),<br>discipline practices<br>(MICS), positive<br>discipline, negative<br>discipline* (DI), family<br>cohesion*, family<br>communication &<br>negative family<br>interactions* (BFFS) | Child-report -<br>Internalizing symptoms<br>& externalizing<br>symptoms*, attention<br>problems (YSR),<br>psychosocial protective<br>factors* (CPPFS).<br>Caregiver-report -<br>internalizing &<br>externalizing<br>symptoms*, attention<br>problems* (CBCL),<br>psychosocial protective<br>factors (CPPFS) |
| Bell et al. (2008)<br>KwaDedangendiale,<br>South Africa              | N = 579;<br>children &<br>adolescents            | HIV-disclosure counselling<br>program with<br>psychoeducation;<br>community; community<br>caregivers               | 10; groups of<br>families                                                                   | Usual services     | General health (GHQ);<br>Well-being (GIWB), child<br>wellbeing (CBCL), AIDS<br>transmission knowledge<br>(AIDSTKS)*, less stigma<br>towards HIV infected<br>people (SS)*,<br>psychological autonomy,<br>involvement, strictness<br>(PaSS), family decision<br>making (FDQ), caregiver<br>monitoring (CMI),<br>communication comfort<br>& frequency (HTA)*,<br>social networks (SC),<br>neighborhood<br>disorganization & social<br>control (ND&CS)                                                                                                                                                                                                                           | Anxiety (RCMA), AIDS<br>transmission knowledge<br>(AIDSTKS), less stigma<br>towards HIV infected<br>people (SS)*, child-<br>report caregiver<br>psychological<br>autonomy,<br>involvement, strictness<br>(PaSS), child-report<br>caregiver monitoring,<br>communication comfort<br>& frequency (CMI)        |
| Betancourt et al.<br>(2017)<br>Kayzona district,<br>Rwanda           | N = 82<br>families;<br>children &<br>adolescents | "Family strengthening<br>intervention"; home<br>delivery setting; counselor                                        | 8-21; individual<br>family                                                                  | Treatment as usual | Child-report - Family<br>connectedness (FC),<br>good parenting (GP).<br>Caregiver-report - Family<br>connectedness (FC),<br>good parenting (GP)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | Child-report -<br>Depression(CES-DC-<br>adapted)*, conduct<br>problems (CDS-<br>adapted), functional<br>impairment (WHODAS-<br>Child), caregiver-report -<br>Depression(CES-DS-<br>adapted)*, conduct<br>problems (CDS-<br>adapted), functional<br>impairment (WHODAS-<br>Child)                            |
| Bhana et al.(2014)<br>Kwazulu-Natal, South<br>Africa                 | N = 65;<br>children &<br>adolescents             | Family-focused<br>psychosocial intervention;<br>primary healthcare setting;<br>lay health worker &<br>psychologist | 6; groups - separate<br>parts for children<br>and parents & joint<br>parents + children.    | Waitlist           | Youth & caregiver<br>communication<br>frequency,<br>communication comfort*<br>(FES), HIV/AIDS illness<br>stigma (Stigma)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | Mental health (SDQ),<br>depression (CDI),<br>adherence to ART<br>(PAIDSCGTG)*, HIV<br>treatment knowledge<br>(HIVTK)                                                                                                                                                                                        |

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Table 1 (continued)

| Study; location                                                             | Sample size; age group                                 | Intervention type; setting; therapist type                                                                                             | Number of sessions; session format                                                                                                         | Comparison group                   | Parent/caregiver/ family outcomes                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | Child outcomes                                                                                                                                                                                                                                                                                                                     |
|-----------------------------------------------------------------------------|--------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Cluver et al. (2018)<br>Eastern Cape Province, South Africa                 | N = 552; caregiver youth dyads; children & adolescents | Sinovuyo Teen Programme parenting program; community setting; lay health workers                                                       | 14; combination of groups of families with some separate sessions for groups of caregivers and separate sessions for groups of adolescents | Hygiene intervention               | Child-report - Physical & emotional abuse, neglect (ICAST), corporal punishment, positive parenting, involved parenting*, poor parental supervision*, inconsistent discipline (APQ). Parent-report - physical & emotional abuse*, neglect (ICAST), corporal punishment*, positive parenting*, involved parenting*, poor parental supervision**, inconsistent discipline (APQ), attitudes to harsh punishment, CR (ICAST-Trial)*, caregiver depression (CESDS)*, parenting stress (PSS)*, caregiver social support (MOS-SSS)* | Child-report - Attitudes to harsh punishment (ICASTTrial), depression & suicidality (CDI and MINI), social support (MOS-SSS), externalizing behaviors (CBCL), alcohol & substance abuse* (WHO-AUDIT), adolescent exposure to violence (SaH). Parent-report - externalizing behaviors (CBCL), adolescent exposure to violence (SaH) |
| Dehghani-Arani, Ali Besharat, Fitton & Aghamohammadi (2018)<br>Tehran, Iran | N = 22 mother-adolescent dyads                         | “Mother–Child-Disease Triangle (MCDT)” attachment-based intervention; hospital; trained doctoral student in clinical health psychology | 10; 4 joint sessions, 3 with mother, 3 with child                                                                                          | Routine care without psychotherapy | Parent and peer attachment* (IPPA), depression* (BDI), parenting stress* (PSI), general health & psychological wellbeing* (GHQ)                                                                                                                                                                                                                                                                                                                                                                                              | General health and psychological wellbeing* (CHQ), depression* (BDI), illness perception* (IPQ)                                                                                                                                                                                                                                    |
| El Khani et al. (2019)<br>West Bank                                         | N = 120; children & adolescents                        | “Caring for Children Through Conflict and Displacement”; community; trained facilitators                                               | 1; groups of caregivers                                                                                                                    | Waitlist                           | Parenting and family adjustment* (PAFAS)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | Internalizing and externalizing problems* (SDQ)                                                                                                                                                                                                                                                                                    |
| Eloff et al. (2014)<br>Tshwane, South Africa                                | N = 390; children                                      | Psychoeducational/skills building, “Kgoro Mmogo”; community setting; community careworkers                                             | 24; combination of groups of families and separate groups for mothers, separate groups for children                                        | Treatment as usual                 | Depression (CES-D) <sup>l</sup> , coping (self, help from others, avoidant) (BC) <sup>l</sup> ; parenting stress (PSI subscales), response to child negative behaviors (CCNES), child internalizing & externalizing* behavior (CBCL), child communication*, daily living skills* & socialization (VABS)<br>None                                                                                                                                                                                                              | Depressive symptoms (CDI), anxiety (RCMAS) <sup>l</sup> , emotional intelligence (BarON EQ-i)                                                                                                                                                                                                                                      |
| Herscovici, Kovalskys & Orellana (2017)<br>Buenos Aires, Argentina          | N = 23; adolescents                                    | “Family Meal Intervention (FMI)”; private office; trained therapist                                                                    | 14–25 dependent on need; family sessions (without siblings)                                                                                | Family Therapy without FMI         | None                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | Weight, percentage of Expected Body Weight, eating disorders (AOS and GOS), menstrual status, eating disorder symptoms and attitudes (EDI-2), psychopathology (SCL-90-R).                                                                                                                                                          |
| Jewkes et al. (2019)<br>Tshwane metropole, South Africa                     | N = 3756; adolescents                                  | “Skhokho Intervention”; school-based; trained facilitator                                                                              | 8; 4 sessions for groups of caregivers & 4 for groups of adolescents                                                                       | Routine care                       | Caregiver child communication*                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | Incidence of interpersonal violence and severe violence, incidence of non-partner rape, childhood trauma*, bullying at school*, communication with a girlfriend/ boyfriend*, delinquency, depression*, gender attitudes <sup>ak</sup>                                                                                              |
| Kuo et al. (2020)<br>Cape Town, South Africa                                | N = 73; adolescents                                    | “Our Family Our Future” resilience-oriented intervention; community setting; trained facilitators with BA or MA                        | 4; 3 group (of dyads) sessions, 1 individualized family session                                                                            | Waitlist                           | Depression (CESD); resilience* (CDRS); communication (PACS)*                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | HIV risk behaviors*; HIV knowledge; risk perception; condom use; self-efficacy;                                                                                                                                                                                                                                                    |

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Table 1 (continued)

| Study; location                                                                                         | Sample size; age group                                  | Intervention type; setting; therapist type                                                                                                                                       | Number of sessions; session format                                                                     | Comparison group   | Parent/caregiver/ family outcomes                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | Child outcomes                                                                                                                                                                                        |
|---------------------------------------------------------------------------------------------------------|---------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------|--------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Li et al. (2014); Li et al. (2011)<br>Anhui Province, China                                             | N = 79; children & adolescents                          | Psychoeducational/skills building, "Together for Empowerment Activities"; community setting; lay health workers                                                                  | 6, with 6 at home activities; combination of group and individual family                               | Treatment as usual | Depressive symptoms for PLH & family members (ZSDS), family functioning (FFS), social support, (MOSSSS)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           | depression* (CESD); resilience (CDRS) Self-esteem* (RSES; overall age groups), perceived parental care* (PBI; 6–12 year olds), problem behavior Youth emotional & behavioral problems (CBCL)*         |
| Li et al. (2017)<br>Anhui Province, China                                                               | N = 475; children & adolescents                         | Psychoeducational/skills building, "Together for Empowerment Activities"; community setting; lay health workers                                                                  | 6, with 6 at home activities; combination of group and individual family                               | Treatment as usual |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                                                                                                                                                                                                       |
| Molleda et al. (2017)<br>Guayaquil, Ecuador                                                             | N = 239; adolescents                                    | "Familias Unidas"; schools and homes; mental health professionals and paraprofessionals (e.g. school counselor and social workers)                                               | 12 sessions; 8 with groups of caregivers & 4 individual family sessions                                | Community Practice | Communication* (PACS), parental monitoring of peers (PRPCS)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | Parent-report conduct problems* (conduct disorder subscale RBPC)                                                                                                                                      |
| Nestadt et al. (2019)<br>Thailand                                                                       | N = 88; caregiver-child & adolescent dyads              | "Collaborative HIV Prevention and Adolescent Mental Health Program (CHAMP+ Thailand)"multiple family group-based intervention; routine HIV care setting; social worker/counselor | 11; separate & combined groups of caregivers & adolescents                                             | Standard Care      | Caregiver mental health* (CESD), knowledge of HIV transmission*, family supervision, youth and caregiver communication*, internalized and externalized HIV stigma*, comfort with HIV disclosure, social support related to child HIV*                                                                                                                                                                                                                                                                                                                                                                                                             | Mental health* (SDQ), depression (CDI), youth ART adherence*, knowledge of HIV transmission*, internalized and externalized HIV stigma, comfort with HIV disclosure                                   |
| O'Callaghan et al. (2014)<br>Haut-Uele Province, Li-May & Kiliwa villages, Democratic Republic of Congo | N = 159; children & adolescents                         | Family-focused psychosocial intervention; community setting; lay health worker                                                                                                   | 8; groups of families                                                                                  | Waitlist           |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   | Child-report - Post-traumatic stress symptoms* (CRIES-8), internalizing symptoms, conduct problems, pro-social behaviour <sup>h</sup> (AYPA). Caregiver-report - Conduct problems <sup>h</sup> (AYPA) |
| Puffer et al. (2016)<br>Migori County, Kenya                                                            | N = 124 families, N = 237 youth, children & adolescents | READY family and church-based intervention (skills building); community setting; trained facilitators                                                                            | 9; groups of families with some sessions for groups of caregivers and groups of adolescents separately |                    | Child report - Negative parenting, positive parenting, parental involvement female caregiver, parental involvement male caregiver, social support female caregiver, social support male caregiver, negative interaction female caregiver, negative interaction male caregiver (APQ, family communication* (PACS). Female caregiver report - Negative parenting, positive parenting, parental involvement, social support, negative interaction (APQ). Male caregiver report - Negative parenting, positive parenting, parental involvement*, social support*, negative interaction (APQ). Overall caregiver report - Family communication* (PACS) | Reduced indicators of future HIV risk (HIVKQ)*, self-esteem (RSES), anxiety (MDASC), depression (CDI, SDQ)                                                                                            |
| Santini & Williams (2017)<br>Brazil                                                                     | N = 40; mother-child & adolescent dyads                 | "Projeto Parceria (Partnership Project)"; child protective services and NGOs; psychologist                                                                                       | 13; 12 individual sessions with mothers, 1 observation session of mother & child                       | Waitlist           | Depression* (BDI), mother-child non-verbal interactions*                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | Depression* (BDI), externalizing and internalizing behaviors* (SDQ)                                                                                                                                   |

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Table 1 (continued)

| Study; location                                               | Sample size; age group                 | Intervention type; setting; therapist type                                                             | Number of sessions; session format                                                                                          | Comparison group   | Parent/caregiver/ family outcomes                                                                                                                                                                                    | Child outcomes                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|---------------------------------------------------------------|----------------------------------------|--------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------|--------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Vreeman et al. (2018)<br>Kenya                                | N = 285; early adolescence             | HIV disclosure intervention; not reported; trained disclosure counselors                               | Not reported; individual & group format                                                                                     | Routine care       | HIV disclosure*                                                                                                                                                                                                      | Internalizing and externalizing behavior (SDQ), depression (PHQ-9)                                                                                                                                                                                                                                                                                                                                                                                                                                         |
| Family-focused interventions: Non-randomized controlled trial |                                        |                                                                                                        |                                                                                                                             |                    |                                                                                                                                                                                                                      |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| Celik et al. (2016)<br>Turkey                                 | N = 66; early childhood                | “First Steps to Success”; school & home; counselor, teacher & parent                                   | 20–30 days teacher with student (first 5 days modeled by the counselor), 6 weeks parents with child (at least 15 min a day) | School as usual    |                                                                                                                                                                                                                      | Social skills and problem behavior* (PKBS)                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| Diken et al. (2011)<br>Turkey                                 | N = 102; children                      | “First Steps to Success”; school & home; counselor, teacher and parent                                 | 20–30 days teacher with student (first 5 days modeled by the counselor), 6 weeks parents with child (at least 15 min a day) | Classroom as usual |                                                                                                                                                                                                                      | Problem behaviors, social skills, and academic competence* (Turkish SSRS)                                                                                                                                                                                                                                                                                                                                                                                                                                  |
| Family-focused interventions: Pre to post studies             |                                        |                                                                                                        |                                                                                                                             |                    |                                                                                                                                                                                                                      |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| Adhikari et al. (2018)<br>Chitwan District, Nepal             | N = 39; children & adolescents         | “The Stepped Care Family Intervention (SCFI)”; school and home (rural); trained psychosocial counselor | 9; 2 sessions groups of teachers, 3 sessions groups of parents, 3 home-visits & 1 follow-up group                           |                    |                                                                                                                                                                                                                      | Behavior problems* (DBIS-N), Child Functional Impairment Scale* (CFIS), parent-report child behavior* (ECBI)                                                                                                                                                                                                                                                                                                                                                                                               |
| Betancourt et al. (2014)<br>Kayzona district, Rwanda          | N = 20 families, 39 children; 7–17 y/o | “Family Strengthening Intervention”; home delivery setting; counselor                                  | Varied according to family’s needs & family size; individual family                                                         |                    | Caregiver-report - Family connectedness* (FC), good parenting* (GP); harsh punishment (MISC4)*, own social support (ISSB)*. Child-report - Family connectedness (FC), good parenting (GP), harsh punishment* (MISC4) | Child report - Perseverance/self-esteem (CDRS), prosocial behavior (PSB), own social support* (ISSB), depression (CESDC), anxiety (YSR), irritability (IRQ), conduct problems (YCPS-RS)*, conduct problems, functional impairment (WHODAS-Child). Caregiver-report - Perseverance/self-esteem* (CDRS), prosocial behavior* (PSB), own social support* (ISSB), depression* (CESDC), anxiety* (YSR), irritability* (IRQ), conduct problems (YCPS-RS), conduct problems, functional impairment (WHODAS-Child) |
| Busse et al. (2021)<br>Indonesia                              | N = 42; adolescents and family members | “Treatnet Family (TF)”; community centers; practitioner                                                | 6; individual family                                                                                                        |                    | Family functioning (FAD)                                                                                                                                                                                             | Friends risk taking behaviors* (SaH), positive and negative attributes (SDQ)                                                                                                                                                                                                                                                                                                                                                                                                                               |
| Buyukkaragoz et al. (2016)<br>Turkey                          | N = 12; children & adolescents         | Counselling & education program; community setting; social worker & paediatric fellow                  | 2–4; individual family                                                                                                      |                    | Health-related quality of life (SF-36), self-esteem (RSES)*, depression (BDI)                                                                                                                                        | Child-report - Health-related quality of life (KINDL), depression (BDI), self-esteem (RSES). Caregiver-report - Health-related quality of life (KINDL)                                                                                                                                                                                                                                                                                                                                                     |
| Cluver et al. (2017)<br>South Africa                          | N = 60; adolescents                    | “Sinovuyo Caring Families Teen Programme”; community; not reported                                     | 10; 4 separate groups of caregivers and adolescents and 6 joint group sessions                                              |                    | Violent and abusive discipline* (ICAST child and parent report), positive parenting* (APQ)                                                                                                                           | Child behavior and emotional problems* (CBCL)                                                                                                                                                                                                                                                                                                                                                                                                                                                              |

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Table 1 (continued)

| Study; location                                                                           | Sample size; age group                                 | Intervention type; setting; therapist type                                                          | Number of sessions; session format                                                                                                               | Comparison group | Parent/caregiver/ family outcomes                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | Child outcomes                                                                                                                                                                                                                                                                                                |
|-------------------------------------------------------------------------------------------|--------------------------------------------------------|-----------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------|------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Cluver et al. (2016)<br>Eastern Cape, South Africa                                        | N = 115; youth caregiver dyads, children & adolescents | “Sinovuyo Teen” parenting program; community setting; child care workers                            | 12; combination of groups of families with some sessions for groups of caregivers and sessions for groups of adolescents                         |                  | Child-report- Physical & emotional abuse and neglect* (ICAST-Child), positive & involved parenting*, poor monitoring & inconsistent discipline* (APQ), social support from caregivers* (SAHA-SSS). Caregiver-report - physical & emotional abuse & neglect* (ICAST-Parent)*, parenting-positive & Involved parenting*, poor monitoring & inconsistent discipline* (APQ), caregiver depression* (CESDS), caregiver substance use* (WHO Assist), parenting stress* (PSS), caregiver social support* (MOS-SSS); Parenting and family adjustment* (PAFAS) | Child report - Rule breaking & aggressive behaviors* (CBCL), depression (CDI)*, witnessing violence (EVS)*, alcohol consumption and drug use (WHO-GSHS), sexual abuse (ICAST-Child). Caregiver-report - Rule breaking & aggressive behaviors** (CBCL), sexual abuse (ICAST-Parent), witnessing violence (EVS) |
| Haar, El-Khani, Molgaard & Maalouf (2020)<br>Afghanistan                                  | N = 72; children & adolescents                         | “Strong Families Program”; community; lay facilitator                                               | 3; 1 session for groups of parents, 1 parallel separate session for groups of caregivers & groups of children, 1 group of families joint session |                  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | Internalizing and externalizing problems* (SDQ)                                                                                                                                                                                                                                                               |
| Maalouf & Campello (2014)<br>Tajikistan, Turkmenistan, Kyrgyzstan, Kazakhstan, Uzbekistan | N = 540 families; children                             | Families & Schools Together (FAST) (skills building); community setting; local trained facilitators | 8; groups of families                                                                                                                            |                  | Family conflict* (FRI of FES)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | Conduct problems* (SDQ)                                                                                                                                                                                                                                                                                       |
| O'Donnell et al. (2014)<br>Moshi & surrounding area, Tanzania                             | N = 64; Children                                       | Trauma-focused CBT; community setting; lay health worker                                            | 12; combination of group sessions (separate for children and caregivers), and individual sessions with child, or with child + parent             |                  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | Child-report - Grief* (GSS)*; PTS* (UCLA PTSD-RI), depression* (SMFQ). Caregiver report - PTS* (UCLA PTSD-RI), depression* (SMFQ), behavioral & emotional well-being (SDQ)*                                                                                                                                   |
| Pereira, D’Affonseca & Williams (2013)<br>Brazil                                          | N = 17; adolescents                                    | “Projeto Parceria (Partnership Project)”;<br>home; not reported                                     | 16; individual sessions                                                                                                                          |                  | Parental competence, parental sense of wellbeing*, parental style* (PSI), child abuse potential inventory (CAP)                                                                                                                                                                                                                                                                                                                                                                                                                                       | Internalizing and externalizing problems* (SDQ)                                                                                                                                                                                                                                                               |
| Puffer et al. (2020)<br>Eldoret, Kenya                                                    | N = 10; adolescents                                    | “Tuko Pamoja (We are together); community; lay counselors                                           | 1–22; family sessions                                                                                                                            |                  | Caregiver mental health* (GHQ), child maltreatment* (MICS), family relationship quality*, couple’s relationship quality*, intimate partner violence and harsh marital interactions*, alcohol use, alcohol-related conflict*                                                                                                                                                                                                                                                                                                                           | Child mental health* (BPM)                                                                                                                                                                                                                                                                                    |
| Rochat et al. (2013; 2015)<br>KwaZulu-Natal, South Africa                                 | N = 281; children                                      | HIV-disclosure counselling program with psychoeducation; community setting; lay health worker       | 6; Individual family                                                                                                                             |                  | Maternal psychological functioning* (GHQ), parenting stress* (PSI-36)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | Internalizing symptoms* and externalizing symptoms* (CBCL)                                                                                                                                                                                                                                                    |
| Smith Fawzi et al. (2012)<br>Central Haiti, Haiti                                         | N = 168 youth, 130 caregivers, children & adolescents  | Psychosocial support intervention; primary healthcare setting; social worker                        | 29; groups of families, with some sessions for groups of caregivers and                                                                          |                  | Depressive symptoms* (HSC-25), role functioning (ACTG Short Form-21), financial                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | Child report - Psychological symptoms* (SDQ), psychosocial                                                                                                                                                                                                                                                    |

(continued on next page)

Table 1 (continued)

| Study; location                                                                              | Sample size; age group                  | Intervention type; setting; therapist type                                                      | Number of sessions; session format                                                            | Comparison group | Parent/caregiver/ family outcomes                                                                                                                       | Child outcomes                                                                                                                                                   |
|----------------------------------------------------------------------------------------------|-----------------------------------------|-------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------|------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                                                                              |                                         |                                                                                                 | some sessions for groups of youth                                                             |                  | social support* (SSA), HIV-related stigma* (HIVRSQ)                                                                                                     | functioning* (SDQ-extended), support from friends*, relatives* (PSR). Caregiver report - Psychological symptoms* (SDQ), psychosocial functioning* (SDQ-extended) |
| Thurman, Nice, Luckett & Visser (2018)<br>Gauteng and Kwa-Zulu Natal provinces, South Africa | N = 178 caregivers, N = 216 adolescents | “Let’s talk” family-centered adolescent HIV prevention program; community; trained facilitators | 19 caregiver & 14 adolescent group sessions, of which six joint caregiver-adolescent sessions |                  | HIV knowledge* (SADHS), caregiver-adolescent sexual risk communication*, adolescent-caregiver connectedness* (IPPA), caregiver mental health* (DASS-21) | HIV knowledge* (SADHS), condom use knowledge*, condom negotiation & sexual refusal self-efficacy (adapted CUSES), mental health* (DASS-21)                       |
| Turkiewicz et al. (2010)<br>Sao Palo, Brazil                                                 | N = 7; adolescents                      | Family-focused treatment for AN; mental healthcare setting; psychiatrist                        | 10–12; individual family                                                                      |                  |                                                                                                                                                         | Adolescent functioning (CGAS) <sup>a</sup>                                                                                                                       |

A-SCAT adapted social capital assessment tool, ACTG SF-21 AIDS clinical trials group short form 21, ADHD-RS-IV DuPaul attention-deficit and hyperactivity disorder inventory, AIDSTKS AIDS transmission knowledge scale, APRS adolescent psychological resilience scale, APBQ academic problem behavior questionnaire, APQ Alabama parenting questionnaire, AQ aggression questionnaire, AR adolescent report, ART antiretroviral therapy, ASIQ aggression and self-injury questionnaire, ASQ or ASQ-C ages & stages questionnaires, AUDIT alcohol use disorders identification test, AYMHS Arab Youth Mental Health Scale, AYPYA African youth psychosocial assessment, BAI Beck anxiety inventory, BarOn EQ-I BarOn emotional quotient inventory-youth version, BASC behavioral assessment system for children-parent rating scales, BC brief-cope, BCS behavioral coding system, BDI Beck Depression Inventory, BFFS Burmese family functioning scale, BMSLSS brief multi-dimensional student’s life satisfaction scale, BPM brief problem monitor, BSID Bayley scales of infants development, BSIMD Bayley scales infant mental development, BSITD Bayley scales of infant and toddler development, BSQ behavior screening questionnaire, CAP child abuse potential inventory, CAPES child adjustment and parent efficacy scale, CARS childhood autism rating scale, CBCL child behavior checklist, CCNES coping with children/s negative emotions scale, CDHS child discipline household survey, CDI child depression inventory, CDRS Connor-Davidson resilience scale, CESD centre for epidemiologic studies depression scale, CESDC center for epidemiologic studies depression scale for children, CESDS center for epidemiologic studies depression scale, CFIS Child Functional Impairment Scale, CGAS children’s global assessment scale, CGI-I clinical global impression - improvement scale, CGI-S clinical global impression - severity scale, CHQ child health questionnaire, CMI caregiver monitoring interview, CPPFS child psychosocial protective factors scale, CPRS Conner’s parent rating scale, CR caregiver report, CRIES-8 impact of events scale, CTS2S Revised Conflict Tactics Scale Short Form, CUSES Condom Use Self-Efficacy Scale, CY-BOCS children’s Yale-Brown obsessive compulsive scale, DASS depression-anxiety-stress scale, DBIS-N Disruptive Behavior International Scale—Nepal version, DBRS disruptive behavior scale for children, DCS Devaluation of Consumer Scale, DI discipline interview, DORBA direct observation of repetitive behaviors in autism, DPICS-III dyadic parent-child interaction coding system, DSRS depression self-rating scale, DSQ parental discipline strategies questionnaire, ECBI Eyeberg child behavior inventory, EDI-2 Eating Disorder Inventory second version, EPDS Edinburgh postnatal depression scale, EVS exposure to violence scale, EurQoL-5 Quality of Life Scale, FAD family assessment device, FC family connectedness, FCI Family Care Indicators, FDQ family decision-making questionnaire, FEI family experiences inventory, FES family environment scale, FFS family functioning scale, FG family grid, FRI family relationship index, GAD-7 General Anxiety Disorder, GHQ general health questionnaire, GIWB global indicator of well-being, GMDS Griffiths mental development scales, GP good parenting, GSS grief screening scale, HIVRSQ HIV related stigma questionnaire, HIVTKQ HIV treatment knowledge questionnaire, HIVKQ HIV knowledge questionnaire, HOME home observation for the measurement of the environment inventory, HSCL Hopkins Symptom Checklist, HTA hard-to-talk about scale, ICAST international society for prevention of child abuse and neglect child abuse screening tool, ICS index of clinical stress, ICQ infant characteristic questionnaire, IES impact of event scale, IRQ irritability questionnaire, IPPA inventory of parental and peer attachment, IPQ illness perception questionnaire, ISPCAN Child Abuse Screening Tool - Trial Caregiver Scale, ISSB inventory of socially supportive behaviors, K10 Kessler Psychological Distress, KA-p or KAP perceived knowledge questionnaire, KBMAQ knowledge of behavioral management of aggression questionnaire, KINDL Kinder Lebensqualität Fragebogen, KMSS Kansas Marital Satisfaction Scale, LSQ learning stress questionnaire, MICS multiple indicator cluster survey, MRHS Morgan-Russell Assessment Schedule, MDASC multi-dimensional anxiety scale for children, MICS multiple indicator cluster survey, MISC4 multiple indicator cluster survey-round, MINI mini international neuropsychiatric interview-kid, MOS-SSS medical outcomes study social support survey, MSPSS multidimensional scale of perceived social support, ND&CS neighborhood disorganization & cohesion scale, OMCI observation of mother & child interaction, PA parents’ activities with children, PACS parent-adolescent communication scale, PARI parental attitude research instrument, PAIDSCG paediatric AIDS clinical trial group, PAFAS the parenting and family adjustment scales, PARI parental attitude research instrument, PAR-Q parental acceptance-rejection questionnaire, PARYC Parenting of Young Children Scale, PAS personality assessment screener, PaSS parenting styles scale, PBC parent behavior checklist, PBI parental bonding instrument, PHQ-9 Patient Health Questionnaire, PIEQ parent involvement in education questionnaire, PKBS preschool and kindergarten behavior scales, PPCA parental perception of child’s academic achievement, PPI parenting practices inventory, PRPGS parent relationship with peer group scale, PS parenting scale, PSAM parental self-efficacy measure, PSB pro-social behavior, PSI perceived stress inventory, PSOC-ES Parenting Sense of Competence Scale-Efficacy Subscale, PSS parenting stress scale, PSR provisions of social relations, PedsQL paediatric quality of life questionnaire, QoLI quality of life inventory, RBS-R repetitive behavior scale - revised, RBPC revised behavior problem checklist, RHS Refugee Health Screener, RMAS revised children’s manifest anxiety scale, RCPM Raven’s coloured progressive matrices, RPS reciprocal parent support, RSES Rosenberg self-esteem scale, SA state anxiety, SADHS South African Demographic and Health Survey, SaH social and health assessment, SSS social support scale, SC social capital, SCARED Screen for Child Anxiety Related Disorders, SSCL-90-R symptom checklist 90 revised, SDQ strength and difficulties questionnaire, SMFQ short mood and feelings questionnaire, SQ strain questionnaires, SR self-report, SRS social relationships scale, SRQ self-regulation questionnaire, SRQ-20 self-reporting, SS stigma scale, SSA social support, SSRS social skills rating system, SSS social support survey, STAI state-trait anxiety inventory, UCLA PTSD-RI UCLA posttraumatic stress disorder-reaction index, VABS Vineland adaptive behavior scales, WEMWBS Warwick-Edinburgh Mental Wellbeing Scale, WHO-5 World Health Organization Five Well-Being Index, WHO-AUDIT WHO alcohol use disorders identification test, WHODAS-child World Health Organization Disability Assessment for Children, YCPSRS Youth conduct problems scale-rwanda short form, YSR youth self-report, ZSDS Zung self-rating depression scale.

\* Significant group difference,  $p < .05$ .

<sup>a</sup> Not significant at post, but different at follow-up.

<sup>a</sup> Infant is defined as less than 2 years, child is defined as between 2 and 12 years, adolescent is defined as between 13 and 18 years.

<sup>b</sup> Comparison group showed greater improvement.



<sup>c</sup> Study also included treatment groups receiving nutritional intervention, but the results reported here relate only to the responsive stimulation intervention.

<sup>d</sup> Effect only significant when combined with an infant nutritional program.

<sup>e</sup> Study included samples from Finland, Greece, UK, Serbia, Cyprus, and Montenegro, but results reported here relate only to Serbia and Montenegro samples (LMICs).

<sup>f</sup> Study included a sample from the USA and one from Mexico, but the results reported here relate only to the Mexico sample (LMICs).

<sup>g</sup> Number of phases, sessions not reported.

<sup>h</sup> No between-group difference, but significant improvement for treatment group over time.

<sup>i</sup> Secondary analysis showed increased attention was significant improved for treatment group.

<sup>j</sup> Treatment group decline.

<sup>k</sup> Outcomes for school arm or school and family arm, but not family arm alone.

objectives ( $n = 79$ ), eligibility criteria ( $n = 59$ ) and enrolment of eligible participants ( $n = 66$ ), as well as consistent implementation of the intervention ( $n = 73$ ), relevant outcome measures ( $n = 75$ ) and accounted for loss to follow up ( $n = 62$ ). However, the majority of studies noted small sample sizes ( $n = 23$ ) did not use multiple measurement timepoints before and after the intervention ( $n = 4$ ), and had limited cultural adaptations ( $n = 26$ ). Representativeness was also low ( $n = 38$ ) as well as blinding of assessors ( $n = 22$ ,  $n = 24$  not reported). The majority of studies did not report on accounting for group effects ( $n = 16$ ).

### 3.3. Parent-focused interventions

#### 3.3.1. Target groups

Parenting interventions ( $n = 34$ ) were delivered to parents or caregivers together or separately ( $n = 17$ ) or they were delivered to mothers only ( $n = 17$ ). The majority of parenting interventions targeted parents with infants ( $n = 14$ ) or children ( $n = 11$ ), and only one intervention targeted adolescents. The remainder targeted both children and adolescents ( $n = 8$ ). Some interventions targeted children with specific presentations such as Attention Deficit Hyperactivity Disorder ( $n = 5$ ) or Autism Spectrum Disorder ( $n = 4$ ).

#### 3.3.2. Intervention type

A range of parenting interventions were evaluated. The most commonly evaluated were “Triple P” parenting programs, Group “Triple P” Level 4 ( $n = 5$  studies), “Triple P” group parenting programme ( $n = 3$  studies), and “Triple P” ( $n = 1$  study). Two studies also evaluated “Psychosocial Stimulation” and the “Caregiver Support Intervention (CSI).” Other interventions were evaluated by only one study.

#### 3.3.3. Outcomes

A range of caregiver and child outcomes were measured and are displayed in detail in Table 1. Caregiver outcomes included mental health and wellbeing outcomes (e.g., depression and anxiety, general wellbeing, psychosocial distress), parenting outcomes (e.g., dysfunctional discipline, parental teamwork, parenting self-efficacy) and family environment (e.g., family functioning, family relationships, stressful family experiences).

Infant, child and adolescent outcomes included mental health and wellbeing (e.g., externalizing and internalizing problems, anxiety and depression, psychosomatic complaints) and developmental outcomes (e.g., mental development, psychomotor development, adaptive behavior, repetitive behavior).

### 3.4. Family-focused interventions

#### 3.4.1. Target groups

Family interventions ( $n = 30$ ) were delivered to whole family units, predominantly including children and caregivers ( $n = 24$ ), with one intervention also including teachers, and some including mothers and children only ( $n = 5$ ). The majority of family interventions broadly targeted parents with either children or adolescents ( $n = 14$ ), or focused on adolescents ( $n = 11$ ), with fewer interventions focusing on children ( $n = 4$ ) and early childhood ( $n = 1$ ). Some interventions targeted

children with specific presentations such as children ( $n = 5$ ) or parents ( $n = 2$ ) with HIV, behavioral difficulties ( $n = 2$ ), or living in poverty or high-risk neighbourhoods ( $n = 2$ ).

#### 3.4.2. Intervention type

A range of family interventions were evaluated. The most commonly evaluated were the “Sinovuyo Caring Families Teen Programme” ( $n = 3$  studies), “First Steps to Success” ( $n = 3$  studies), “Family Strengthening Intervention (FSI)” ( $n = 2$ ), “Together for Empowerment Activities (TEA)” ( $n = 2$ ), “Families & Schools Together (FAST)” ( $n = 2$ ), and “Projeto Parceria (Partnership Project)” ( $n = 2$ ). The remainder were evaluated by single studies.

#### 3.4.3. Outcomes

Caregiver outcomes included mental health and wellbeing (e.g., general health, wellbeing, quality of life), HIV knowledge and stigma, parenting practices (e.g., positive parenting, physical and emotional abuse and neglect, parenting stress) and family and community dynamics (e.g. family cohesion, communication, functioning).

Child and adolescent outcomes included mental health and wellbeing (e.g. behavior problems, functional impairment, internalizing and externalizing problems) and HIV knowledge and stigma.

### 3.5. Common practice element coding

Out of all included interventions, 43 had evidence of an effect on a child or adolescent mental health or wellbeing outcome ( $n = 20$  parenting interventions,  $n = 23$  family interventions), including for anxiety, depression, trauma, behavior and psychosocial wellbeing. Twenty-five interventions were coded using manuals (3 were publicly available), and the remaining interventions were coded using the description in the study and additional materials when available (such as publications on the intervention development). Interventions were coded using PracticeWise with the addition of *homework* and *building a family identity*, and subcodes for some practice elements like communication, to identify whether the activity was directed from therapist to parent, therapist to child, or both. In total, out of 152 codes and subcodes, 107 were endorsed by at least one included intervention. Fig. 1 displays the components endorsed by at least 15 interventions. The most common practice elements, occurring in more than 50% of interventions, were caregiver psychoeducation, caregiver coping, accessibility promotion, homework, communication skills (therapist to parent), insight building (therapist to parent), relationship/rapport building and differential reinforcement.

[Fig. 2 insert here].

### 3.6. Theoretical underpinning and mechanisms of change

Table 2 displays the aims and mechanisms as reported by the authors of the studies on the 43 interventions with evidence of an effect on a child or adolescent mental health or wellbeing outcome. Seven parenting interventions were based on cognitive or behavioral theory, largely aiming to improve parenting practices and reduce challenging behavior. Four interventions were based on positive parenting, aiming to improve parenting and parent-child relationships. One intervention

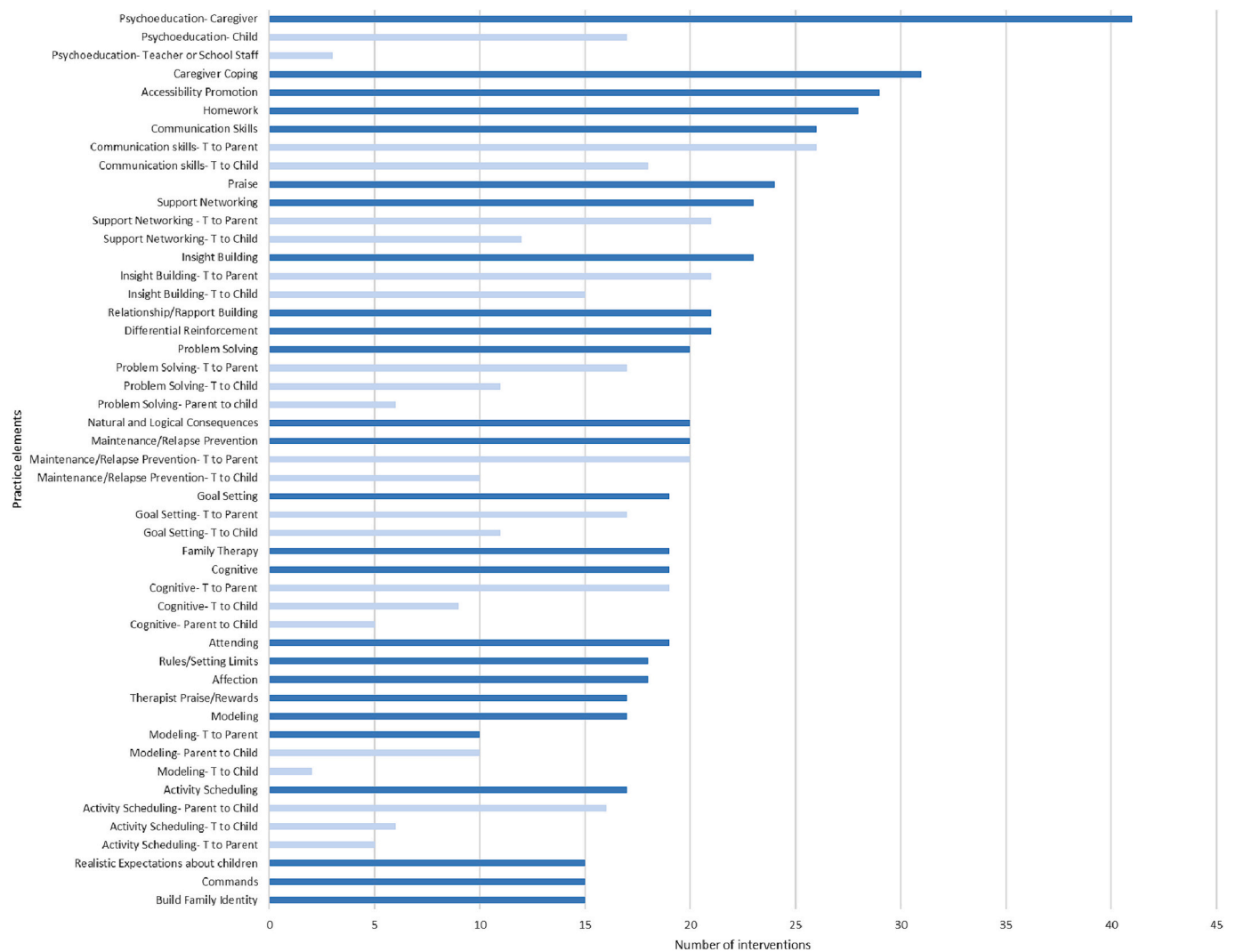


Fig. 2. Practice element components of parenting or family program effective for a child or adolescent mental health outcome, coded using PracticeWise.

was based on education theory to reduce punitive parenting practices, and one intervention was based on Piagetian theory to improve parent knowledge of child rearing. Two interventions were based on an integration of theories; mindful parenting [34] integrated social cognitive theory and mindfulness, while the psychoeducational intervention by Zu et al. [35] integrated trauma-focused cognitive behavioral theory and attachment theory. The remainder of parenting interventions did not report their theoretical basis. Only one study measured mediating factors; Miller et al. [22] found a significant mediating effect of caregiver distress on harsh parenting for the “Caregiver Support Intervention (CSI)” in Lebanon.

Many family interventions were based on intervention specific theories of change ( $n = 7$ ), linking knowledge and skills to family relationships and functioning (“Happy Families” and “Let’s Talk”), parent-child interactions to mental health (“Family Strengthening Intervention” and the HIV-disclosure counselling program), gender-based drivers of interpersonal violence to parenting and communication (“Skhokho Intervention”), or communication to parenting and adolescent mental health (“READY”). Other interventions drew on a range of theories, from Social Action Theory ( $n = 2$ ), to Attachment Theory ( $n = 1$ ), and Family Systems Theory ( $n = 1$ ). Two studies measured mediating factors. Cluver et al. [36] found a significant mediating effect of perceived social support on adolescent aggression and caregiver violent discipline for the “Sinovuyo Caring Families Teen Programme” in South Africa, and Molléda et al. [37] found a significant mediating effect of communication

for the “Familias Unidas” intervention on conduct problems in Ecuador, but no effect of parental monitoring.  
[Table 2 insert here].

3.7. Implementation component analysis

3.7.1. Acceptability

Effective interventions for child and adolescent mental health or wellbeing reported high satisfaction from participants based on qualitative feedback or self-report ratings ( $n = 17$ ), with specific positive feedback related to comfort with the facilitator ( $n = 6$ ) and cultural appropriateness of the materials ( $n = 3$ ).

3.7.2. Attendance

Twenty interventions reported session attendance of 90% or more, while 13 reported attendance levels between 60 and 89%. Only 3 reported attendance levels below 60%. Gender and age differences were noted for 4 interventions; “Let’s Talk” reported higher attendance from parents than adolescents while the “Sinovuyo Caring Families Teen Programme” found the inverse with better attendance by children than parents. “READY” and the “Caregiver Support Intervention (CSI)” reported higher attendance by mothers than fathers.

3.7.3. Adoption

Evidence for the adoption of an intervention into existing services

**Table 2**Theoretical basis, aims and mechanisms of interventions effective for a child or adolescent mental health outcome ( $n = 43$ ).

| Theoretical basis                 | Parenting interventions                                                                                                                                                                          |                                                                                                                                                                                                                            |                                                                                                                       |
|-----------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------|
|                                   | Program (study/ies)                                                                                                                                                                              | Theoretical framework or mechanism(s); intervention aims                                                                                                                                                                   | Mediational analysis                                                                                                  |
| Cognitive/<br>behavioral          | Quality of Life Therapy (Abedi & Vostanis, 2010)                                                                                                                                                 | <b>Cognitive therapy</b> techniques aiming to change core quality of life concepts, attitudes, skills, strengths and positive schemas.                                                                                     | NR                                                                                                                    |
|                                   | Parent-delivered behavioral intervention (Bello-Mojed et al., 2016)                                                                                                                              | <b>Behavioral theory</b> ; aiming to reduce challenging behavior.                                                                                                                                                          | NR                                                                                                                    |
|                                   | “Helping Challenging Children” (Fayyad et al., 2010)                                                                                                                                             | <b>Behavioral theory</b> ; aiming to reduce externalizing behavior.                                                                                                                                                        | NR                                                                                                                    |
|                                   | Parent-child interaction therapy (PCIT) (Matos et al., 2006)                                                                                                                                     | <b>Behavioral therapy, play</b> ; aiming to reduce behavior problems.                                                                                                                                                      | NR                                                                                                                    |
|                                   | Family-based management (Shiri, Pouretmad, Fathabadic & Narimanid, 2020)                                                                                                                         | <b>Applied behavioral theory</b> ; aiming to improve parents’ skills to prevent or to overcome behavioral excesses.                                                                                                        | NR                                                                                                                    |
|                                   | Parenting intervention (Shaw et al., 2021)                                                                                                                                                       | <b>Culturally adapted cognitive behavioral therapy</b> , aiming to improve strengthening parenting self-efficacy, increasing positive parenting practices, supporting family functioning, and enhancing parent well-being. | NR                                                                                                                    |
| Educational                       | Behavioral family therapy (Yu et al., 2015)                                                                                                                                                      | <b>Behavioral theory</b> ; aiming to improve caregivers positive parenting and child compliance to caregiver instructions.                                                                                                 | NR                                                                                                                    |
|                                   | “STAR” parenting psychoeducation program (Solis-Camara et al., 2000)                                                                                                                             | <b>Preventive, educational philosophy building on family strengths</b> ; aiming to reduce punishment and improve child behavior.                                                                                           | NR                                                                                                                    |
| Piagetian theory                  | “Psychosocial Stimulation” (Gardener et al., 2005; Hamadani et al., 2006)                                                                                                                        | <b>Piagetian theory</b> ; aims to improve children’s growth and development, mothers’ knowledge of child rearing, and mother-child interactions.                                                                           | NR                                                                                                                    |
| Positive parenting                | Group “Triple P” Level 4 (Aghebbati et al., 2020; Aghebbati, Gharraee, Shoshtari & Gohari, 2013; Ozbek, Gencer & Mustan, 2019; Ozyurt, Gencer, Ozturk & Ozbek, 2016; Yusuf, Gonka & Aynur, 2019) | <b>Behavioral theory, positive parenting</b> , aiming to prevent severe behavioral, emotional, and developmental problems in children, and improve parenting styles.                                                       | NR                                                                                                                    |
|                                   | “Triple P” group parenting programme (Guo et al., 2016; Mejia et al., 2015; Sumargi et al., 2015)                                                                                                | <b>Positive parenting</b> ; aiming to provide moderate to high intensity intervention to improve parent-child interaction and parenting skills.                                                                            | NR                                                                                                                    |
|                                   | “Triple P” (Kousha & Abbasi Kakrodi, 2019)                                                                                                                                                       | <b>Positive parenting</b> , aiming to improve mother-child interaction to enhance quality of life.                                                                                                                         | NR                                                                                                                    |
|                                   | “Caregiver Support Intervention (CSI)” (Miller et al., 2022; Miller et al., 2020)                                                                                                                | <b>Positive parenting, culturally integrated approach</b> ; aiming to strengthen parenting by lowering stress and strengthening mental health and wellbeing of primary caregivers.                                         | Significant mediating effect of caregiver distress.                                                                   |
| Integration of theories           | Mindful parenting (Dehkordian et al., 2017)                                                                                                                                                      | <b>Social cognitive theory, mindfulness</b> ; aiming to improve mindful parenting and social cognitive deficits.                                                                                                           | NR                                                                                                                    |
|                                   | Psychoeducational intervention (Zu et al., 2019)                                                                                                                                                 | <b>Trauma-focused cognitive behavioral theory, attachment theory</b> ; aiming to improve the attitude of mothers.                                                                                                          | NR                                                                                                                    |
| NR                                | Psychosocial intervention (Dybdahl & Pasagic, 2000)                                                                                                                                              | NR; aiming to promote mother’s understanding of her children and her own role, her well-being, her self-confidence and her ability to care for her children.                                                               | NR                                                                                                                    |
|                                   | Psychoeducation (Jordans et al., 2013)                                                                                                                                                           | NR; aiming to improve child mental health and family social support.                                                                                                                                                       | NR                                                                                                                    |
|                                   | “SANAD (SUPPORT)” Parenting Intervention (Lakkis et al., 2020)                                                                                                                                   | NR; aiming to positively influence different aspects of parenting such as disciplines and behavior.                                                                                                                        | NR                                                                                                                    |
|                                   | “Systematic Training for Effective Parenting (STEP)” (Sangawi, Adams & Reissland, 2018)                                                                                                          | NR; aiming to improve parenting styles and decrease the level of parental stress.                                                                                                                                          | NR                                                                                                                    |
|                                   | Parenting intervention (Shi et al., 2020)                                                                                                                                                        | NR                                                                                                                                                                                                                         | NR                                                                                                                    |
| Family interventions              |                                                                                                                                                                                                  |                                                                                                                                                                                                                            |                                                                                                                       |
| Theoretical basis                 | Program (study/ies)                                                                                                                                                                              | Theoretical framework or mechanism(s); intervention aims                                                                                                                                                                   | Mediational analysis                                                                                                  |
| Attachment theory                 | “Mother-Child-Disease Triangle (MCDT)” (Dehghani-Arani, Ali Besharat, Fitton & Aghamohammadi, 2018)                                                                                              | <b>Attachment theory, stress-focused model of disease behavior attachment</b> ; aiming to improve the mother-child relationship.                                                                                           | NR                                                                                                                    |
| Behavioral/social learning theory | “Caring for Children Through Conflict and Displacement” (El Khani et al., 2019)                                                                                                                  | <b>Behavioral theory, social learning theory</b> ; aiming to build parenting confidence and self-regulatory skills, and enhance child and family psychological well-being.                                                 | NR                                                                                                                    |
|                                   | “Sinovuyo Caring Families Teen Programme” (Cluver et al., 2016; Cluver et al., 2017; Cluver et al., 2018)                                                                                        | <b>Social learning theory</b> ; aimed at reducing parenting stress and improving adolescent outcomes.                                                                                                                      | Significant mediating effect of perceived social support.                                                             |
| Cognitive/behavioral              | Trauma-focused Cognitive Behavioral Therapy (CBT) (O’Donnell et al., 2014)                                                                                                                       | <b>Trauma-focused cognitive behavioral theory</b> ; aiming to improve child grief and post-traumatic stress, depression and wellbeing.                                                                                     | NR                                                                                                                    |
|                                   | “Projeto Parceria (Partnership Project)” (Pereira, D’Affonseca & Williams, 2013; Santini & Williams, 2017)                                                                                       | <b>Cognitive behavioral</b> ; aiming to teach mothering skills, and to prevent behavioral problems in children.                                                                                                            | NR                                                                                                                    |
| Eco-developmental theory          | “Familias Unidas” (Molleda et al., 2017)                                                                                                                                                         | <b>Eco-developmental theory</b> ; aiming to improve family functioning (parent-adolescent communication and parental monitoring) and prevent adolescent conduct problems.                                                  | Significant mediating effect of parent-adolescent communication, no mediating effect of parental monitoring of peers. |
| Family systems theory             | “Tuko Pamoja (We are together)” (Puffer et al., 2020)                                                                                                                                            | <b>Solution-focused and family systems theories</b> ; create a positive family environment supportive of positive child and adult outcomes.                                                                                | NR                                                                                                                    |

(continued on next page)

Table 2 (continued)

| Family interventions                     |                                                                                                              |                                                                                                                                                                                                                                                                                                                                                |                      |
|------------------------------------------|--------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------|
| Theoretical basis                        | Program (study/ies)                                                                                          | Theoretical framework or mechanism(s); intervention aims                                                                                                                                                                                                                                                                                       | Mediational analysis |
| Information-Motivation Behavioral theory | "Our Family Our Future" (Kuo et al., 2020)                                                                   | <b>Information-Motivation-Behavioral theory, adolescent development theory;</b> aiming to identify a family's unique strengths, encourage families to map their family's unique goals, and to create family strategies to overcome their specific challenges.                                                                                  | NR                   |
| Intervention specific theory of change   | "Happy Families" (Annan et al., 2016; Puffer et al., 2017)                                                   | <b>Intervention specific theory of change</b> linking the intervention through acquisition of both knowledge and skills to improved parent-child interactions and family functioning; aiming to improve parenting practices, family functioning, and child behavior, psychosocial wellbeing, and resilience.                                   | NR                   |
|                                          | "Family Strengthening Intervention" (Betancourt et al., 2017; Betancourt et al., 2014)                       | <b>Intervention specific theory of change</b> linking communication and overall parent-child relationships to mental health problems; aiming to improve caregiver-child relationships and family protective constructs in order to improve child mental health.                                                                                | NR                   |
|                                          | "Skhokho Intervention" (Jewkes et al., 2019)                                                                 | <b>Education theory, intervention specific theory of change</b> linking drivers of interpersonal violence; aiming to reduce violence, patriarchal gender attitudes and strengthen parenting and communication.                                                                                                                                 | NR                   |
|                                          | "READY" (Puffer et al., 2016)                                                                                | <b>Intervention specific theory of change</b> linking communication to better parenting and social support, and youths' mental health and HIV risk behavior; aiming to improve family communication and HIV-related skills and beliefs.                                                                                                        | NR                   |
|                                          | HIV-disclosure counselling program (Rochat et al., 2015; Rochat et al., 2013)                                | <b>Intervention specific theory of change</b> linking the quality of the parent-child relationship to children's psychological wellbeing; aiming to improve maternal psychological functioning, parenting stress and children's psychological functioning.                                                                                     | NR                   |
|                                          | "Let's talk" Thurman, Nice, Luckett & Visser (2018)                                                          | <b>Cognitive behavioral, intervention specific</b> theory of change; aiming to improve core HIV knowledge and behavioral skills, caregiver and adolescent mental health, relationships, and parenting practices.                                                                                                                               | NR                   |
|                                          | "Strong Families Program" (Haar, El-Khani, Molgaard & Maalouf, 2020)                                         | <b>Intervention specific logic model;</b> aiming to improve child behavior and family functioning.                                                                                                                                                                                                                                             | NR                   |
| Social action theory                     | "Together for Empowerment Activities (TEA)" (Li et al., 2017; Li et al., 2014; Li et al., 2011)              | <b>Social action theory;</b> aiming to improve mental health, family functioning and social support.                                                                                                                                                                                                                                           | NR                   |
|                                          | "Collaborative HIV Prevention and Adolescent Mental Health Program (CHAMP+ Thailand)" (Nestadt et al., 2019) | <b>Social action theory;</b> aiming to promote mental health and prevent sexual and drug use risk behavior.                                                                                                                                                                                                                                    | NR                   |
| Social cognitive theory                  | Psychosocial support intervention (Smith Fawzi et al., 2012)                                                 | <b>Social cognitive theory;</b> aiming to improve coping skills and social support.                                                                                                                                                                                                                                                            | NR                   |
| Stepped care                             | "The Stepped Care Family Intervention (SCFI)" (Adhikari et al., 2018)                                        | <b>Stepped care model;</b> aiming to support teachers and families in dealing with children/adolescents presenting with, or at risk for, behavior problems                                                                                                                                                                                     | NR                   |
| Integration of theories                  | "Families and Schools Together (FAST)" (Maalouf & Campello, 2014; Macdonald & Doostgharin, 2013)             | <b>Social ecological theory, family systems theory, family stress theory, attachment theory, social learning theory, group theory, risk and protective factors theory, social capital theory and the new brain research;</b> aiming to build protective factors for children to increase their resilience against stress and their well-being. | NR                   |
| NR                                       | "First Steps to Success" (Celik et al., 2016; Diken et al., 2011)                                            | NR; aims to improve social skills and problem behaviors.                                                                                                                                                                                                                                                                                       | NR                   |
|                                          | Family-focused psychosocial intervention (O'Callaghan et al., 2014)                                          | NR; improve the mental health and psychosocial outcomes.                                                                                                                                                                                                                                                                                       | NR                   |
|                                          | Family-focused treatment (Turkiewicz et al., 2010)                                                           | NR; aims to treat Anorexia Nervosa.                                                                                                                                                                                                                                                                                                            | NR                   |

NR not reported.

included support from, or integration of, a intervention with governmental health services or UN agency programming. This was reported by only 4 interventions.

### 3.7.4. Appropriateness

The contextual relevance of the intervention to the population and setting was described in terms of interviews with families or local representatives to inform the development of the program, or adaptations to pre-existing interventions ( $n = 16$ ), minor adaptations to pre-existing intervention material such as translating to the local language or dialect, or amending names in the manual to locally relevant names ( $n = 7$ ),

development of the intervention led by a locally based team ( $n = 4$ ), and/or informed by a desk review of existing interventions ( $n = 3$ ).

### 3.7.5. Feasibility

Studies reported on facilitation, format and duration of the intervention and sessions, supervision and training. A greater number of interventions were delivered by non-specialist ( $n = 16$ ), compared to specialist facilitators ( $n = 14$ ). The vast majority of interventions were delivered in group or multi-family formats ( $n = 22$ ), with few interventions delivered to individual parenting teams or families ( $n = 9$ ). Some interventions also combined individual and groups sessions ( $n =$

11). Most interventions consisted of 8–10 sessions ( $n = 15$ ) or more ( $n = 15$ ), with only 11 interventions consisting of 7 or less sessions. Most interventions had sessions of 1.5 to 2 h ( $n = 10$ ), with only 1 intervention delivering sessions lasting less (1 h) and 1 more (3 h). Specialist supervision and brief training by a specialist was used by all interventions that reported on supervision and training ( $n = 21$  and  $n = 14$  respectively), which included train the trainer models and snowballing. In addition, 2 interventions reported using mock sessions as part of the training.

### 3.7.6. Fidelity

Manuals or guidance materials were used by 17 interventions. To measure fidelity, sessions were observed using a structured checklist ( $n = 11$ ) or facilitators filled out self-report questionnaires ( $n = 8$ ).

### 3.7.7. Implementation costs

In addition to the running costs of staff (facilitators, supervisors and trainers) and facilities for center or hospital-based interventions, some interventions also included reimbursement for transportation costs ( $n = 11$ ), as well as a meal or refreshments during or after sessions ( $n = 5$ ).

### 3.7.8. Intervention complexity

Factors related to intervention complexity were not reported by any study.

### 3.7.9. Penetration

Factors related to penetration in services were not reported by any study.

### 3.7.10. Reach

In terms of the reach of the intervention to the target population, two main barriers were reported; that more male than female child participants attended sessions ( $n = 4$ ), and less fathers than mothers engaged with the intervention or attended sessions ( $n = 8$ ).

Full details of the implementation component analysis can be found in Supplementary Table 3.

### 3.7.11. Sustainability

Factors related to sustainable implementation were not reported by any study.

### 3.7.12. Challenges and lessons learned

The most commonly reported challenge was difficulty in engaging fathers to improve attendance, and fathers preventing mothers and children's attendance ( $n = 6$ ). Authors recommended framing interventions as parenting support and engaging men early on, as well as scheduling sessions around working hours. Equally as common was the challenge of the major adversities facing families, such as needing shelter, food, and healthcare ( $n = 6$ ). Authors recommended offering other types of interventions, such as nutrition, family planning, basic health care, economic support and income-generating projects, to holistically address these adversities. Additionally, it was recommended that the content of interventions should reflect these challenges, as some activities may be inappropriate for the level of adversity families are facing.

Other challenges and lessons learned included helping to prevent absenteeism by taking into account the demands on caregivers in their everyday life, including childcare for infants and younger children ( $n = 5$ ); using visual aids to support the engagement of participants with low literacy ( $n = 3$ ); addressing perceptions of monetary compensation by communities which can cause animosity – though some families also recommended increasing monetary compensation to support attendance ( $n = 3$ ); delivering sessions in community or home settings to increase accessibility ( $n = 3$ ); and delivering interventions drawing on the expertise of local non-specialist facilitators from the community who are better able to engage with families ( $n = 2$ ). Though challenges of

requiring additional extensive training and supervision support for non-specialists were reported, as well as increased experiences of discomfort around sensitive topics (such as sexual health), authors reported that the benefits outweighed the costs. One intervention reported limitations in feasibility and reach because of requiring a specialist provider. Additionally, one intervention reported that families shared content with other families in their communities (such as on the bus, at their churches) and recommended using snowball recruitment through families themselves; and one reported having to suspend sessions during escalating community violence, riots, and flooding. They recommended making it clear that the intervention is not associated with any group or political party, and avoiding home visits for staff safety. Lastly, authors of one study recommended phone calls between sessions and make-up sessions for missed sessions, to maintain momentum and engagement in the program.

## 4. Discussion

This systematic review aimed to identify the state of the evidence, intervention components commonly included, theoretical mechanisms of change, and implementation considerations of parent and family interventions for child and adolescent mental health and wellbeing in low- and middle-income countries. Evidence was hampered by small sample sizes and high heterogeneity across studies. However, evaluations indicate some promising preliminary evidence of effectiveness for child or adolescent mental health and wellbeing, across populations and contexts. This provides initial evidence for the relevance of parenting and family interventions for improving childhood mental health, in line with past research and recommendations that argue that mental health service providers need to address the wider social ecology of children and adolescents to protect and promote their mental health and reduce distress [13,38]. We found more than double the studies and interventions in just 5 years since the last review, indicating an acceleration of research on these. Out of the 80 studies of 64 different parent or family interventions, potential positive effects were reported for 43 interventions with child or adolescent mental health or wellbeing outcomes. Only 12 studies out of 80 were conducted in humanitarian settings, demonstrating that research in these settings remains a major priority. Almost half the studies were randomized control trials and scored medium to high for methodological quality. Included outcomes ranged from child and adolescent mental health, wellbeing, and development, to parenting, family environment, and community dynamics. In addition to evidence of effectiveness, we identified core practice elements of identified effective interventions, theoretical underpinnings, implementation commonalities across effective interventions, and key lessons learned.

The core practice elements of effective parenting and family interventions identified in this review - caregiver psychoeducation, caregiver coping, accessibility promotion, homework, communication skills, insight building, relationship/rapport building and differential reinforcement – reflect the importance of accessibility, practice, communication and relationships, as well as behavior theory informed parenting practices. Whilst parenting interventions have traditionally been grounded in teaching parenting skills, such as differential reinforcement, our findings indicate the importance of also addressing parent wellbeing and communication within the family. This is supported by some evidence of mediating effects of communication [37], social support [36] and caregiver distress [22] on treatment outcomes for child mental health. This is in line with theoretical assumptions that improvement in caregiver distress and family functioning can improve child distress [9]. However, very few studies measured potential mediators of change.

Additionally, the importance of accessibility as a core practice element was highlighted, which is defined by PracticeWise as ‘any strategy used to make services convenient and accessible or to proactively enhance treatment participation.’ Interventions were

predominantly community-based within the local area of families, and some interventions also covered transport costs and food. This is particularly relevant for low-income families and in low resource settings, as evidence-based interventions that are not accessible have little use. Hospital and clinic based mental health services have long been criticized for lack of accessibility to those most in need [39], and our findings demonstrate the positive impact of efforts to move into community-based service delivery. Finally, the importance of homework to practice learned skills between sessions is also in line with past research on specialist interventions, as it helps to apply learning in everyday life [40].

Despite the systemic nature of parenting and family work, only two interventions drew on systems theory [41,42]. Instead, interventions drew on a range of broader social ecological or interpersonal theories, or on cognitive behavioral theory applied to parenting practices. Whilst MHPSS informed by cognitive behavioral theory has some good evidence of effectiveness for child and adolescent mental health, it has been criticized for neglecting social determinants of mental health [11] and interpersonal processes underlying many mental disorders [9]. Our findings indicate that integration of cognitive behavioral content with systemic and interpersonal content is not only possible, but may be effective in addressing both children and adolescents' familial environment and caregivers' wellbeing, as well as child and adolescent mental health and wellbeing.

In terms of implementation, we found commonalities across effective interventions, as well as rich feedback on challenges and lessons learned that can be used to pre-empt difficulties when implementing parenting or family interventions. While research in high-income countries has a strong evidence-base for specialist parenting interventions and family therapy [3], our review in low-and middle-income countries indicated feasibility of delivering effective whole family interventions in low-resource settings and by non-specialist providers. Local non-specialists were reported as having vital local knowledge and community engagement, the value of which far outweighed the time and costs needed to invest in training and supervision of non-specialists through task sharing models. Most interventions used manuals or guidance materials, training by a specialist, regular supervision sessions during implementation and fidelity assessments through observation or self-report. This is in line with increasing evidence of the importance of supportive supervision, particularly in humanitarian and conflict-affected settings [43]. In terms of challenges and lessons learned, the need to integrate MHPSS into wider health and social care, or across humanitarian sectors, is echoed by the increasing acknowledgment of the benefits of joined up and holistic care [8]. Recent advances in integrating MHPSS with health, nutrition and education have been made with the development and piloting of a Minimum Services Package for MHPSS [44], though this is specific to humanitarian emergencies. Such integrated care packages are clearly needed across the board in low and middle – and indeed in high – income countries.

Many authors described challenges in engaging fathers in interventions, both in terms of attendance but also participation and motivation. Research has shown that fathers are just as impactful as mothers in building attachment relationships and protecting the mental health of children [45]. Research in high-income countries has long highlighted the need for father' engagement in parenting and family interventions [46]. In our review, authors recognized the importance of fathers and the parenting team, but also faced challenges particularly in terms of communicating the importance of father' attendance – or whole family attendance – to family members, and in scheduling sessions at times that were accessible for working men and boys. The need for flexible working and early engagement with fathers was strongly recommended by authors. Finally, in terms of accessibility and sustainability, we found that only a few studies reported that interventions were implemented beyond the end of the project. The need to integrate interventions into existing services or programs in order to improve the reach and scalability of interventions for affected populations is an

increasing priority globally [47]. Authors did describe, however, delivery at the community-level in order to improve accessibility, though some authors noted the need for younger children to be accommodated to address accessibility barriers even at this level. Previous research has identified practical barriers – such as distance, or childcare – as the main reasons for declining MHPSS services [48], demonstrating the vital importance of accessibility.

In terms of methodologies, whilst a good proportion of studies followed an RCT design (48 out of 80 studies), limitations of low sample size, not using multiple time points, and limited cultural adaptations of interventions demonstrate the need to improve methodologies in this field. Large enough sample sizes can be difficult to achieve given the major challenges of low resource settings, particularly during humanitarian emergencies. Disruptions to assessments and intervention implementation due to spikes in violence, security risks, roadblocks and curfews all pose major barriers to recruitment and retention of participants. In addition, the costs of a large trial makes it necessary to obtain funding, which is scarce in mental health research in LMICs [49]. Relatedly, measuring outcomes across multiple time points can be challenging, as it requires higher human resources and staff costs. Multiple time points could provide much better evidence for mechanisms of change, which are crucial to understanding underlying processes that generate therapeutic change [21] – particularly important for low intensity non-specialist interventions in 'task sharing models of mental health care, and an MHPSS SET priority research question identified by the IASC Reference Group on Mental Health and Psychosocial Support in Emergency Settings [19]. Given the low level of evidence we found on testing of mechanisms of change in this review (3 out of 80 studies) this remains a clear research gap. Finally, in terms of cultural adaptation, only a small number of interventions were locally developed, or adapted to the context and culture beyond just translating to the local language and changing names in case scenarios. More substantial and evidence-informed cultural adaptation is needed [50].

#### 4.1. Strengths and limitations

This systematic review benefits from a comprehensive methodology including a grey literature search, both an analysis of effectiveness *and* implementation, and a focus on under-researched family and parenting MHPSS interventions. However, in order to have a comprehensive understanding of available evidence in this emergent field of work, pre-post studies were included and this may increase the risk of bias. Effectiveness results should therefore be interpreted with caution. In addition, most interventions were not assessed in replication studies in different contexts, therefore the evidence-based for specific interventions, and for which interventions work more or less well, is not well developed. The review also focused on parents and families, but future reviews would benefit from widening out search criteria to include the context around the family, such as programs addressing community cohesion and isolation. Due to high heterogeneity it was not possible to conduct a meta-analysis, and early childhood development interventions were excluded from practice and implementation component analyses. We strongly recommend replication studies across contexts of promising interventions in order to support meta-analyses and more robust conclusions about intervention effectiveness and feasibility. In addition, small sample sizes identified in the quality assessment indicate that some studies may be under-powered, though power calculations were often not reported. Reporting of power calculations are needed to better inform the strength of evidence. Finally, the small number of studies that ran mediational analyses means that evidence on mechanisms of change is limited.

#### 4.2. Implications and conclusions

Findings from this review of family and parenting interventions in low- and middle-income countries has implications for broadening the



scope of current MHPSS programming to address the wider social and interpersonal environment of children and adolescents. We found promising evidence for the effectiveness of family and parenting interventions for a range of child, adolescent, caregiver and family outcomes, as well as the feasibility of delivering such interventions through supervised and trained non-specialists to improve their accessibility and reach. Our findings suggest a critical role of parental and family relationships and communication in promoting the mental health children and adolescents. The accessibility of interventions could be further explored in future research using implementation science approaches to understand what kinds of service models can help to support such work. For example, how to balance facilitator safety and work-life balance with family needs for out of hours appointments, how to better engage with fathers or how to scale-up or integrate interventions into existing services. Future research would also benefit from more high quality RCTs, replications across contexts, and measurement of more clearly defined mechanisms. Our findings indicate that systems theory has not been widely drawn on to inform interventions that are systemic by nature – given the relevance of systems theories, such as liberation psychology [51], the translation of such theoretical mechanisms into systemic MHPSS may help to further strengthen their impact. The wealth of implementation data derived from this review highlights the benefits of sharing learning and future studies are strongly encouraged to report on implementation challenges and lessons learned. In this way, the MHPSS field will be better able to improve the accessibility and acceptability of interventions. Finally, further research is needed to build evidence on how to build up systems of care that are sustainable and scalable, whilst maintaining quality.

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## Declaration of competing interest

The authors declare no conflict of interests.

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## Appendix A. Supplementary data

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